

The Cranial Base

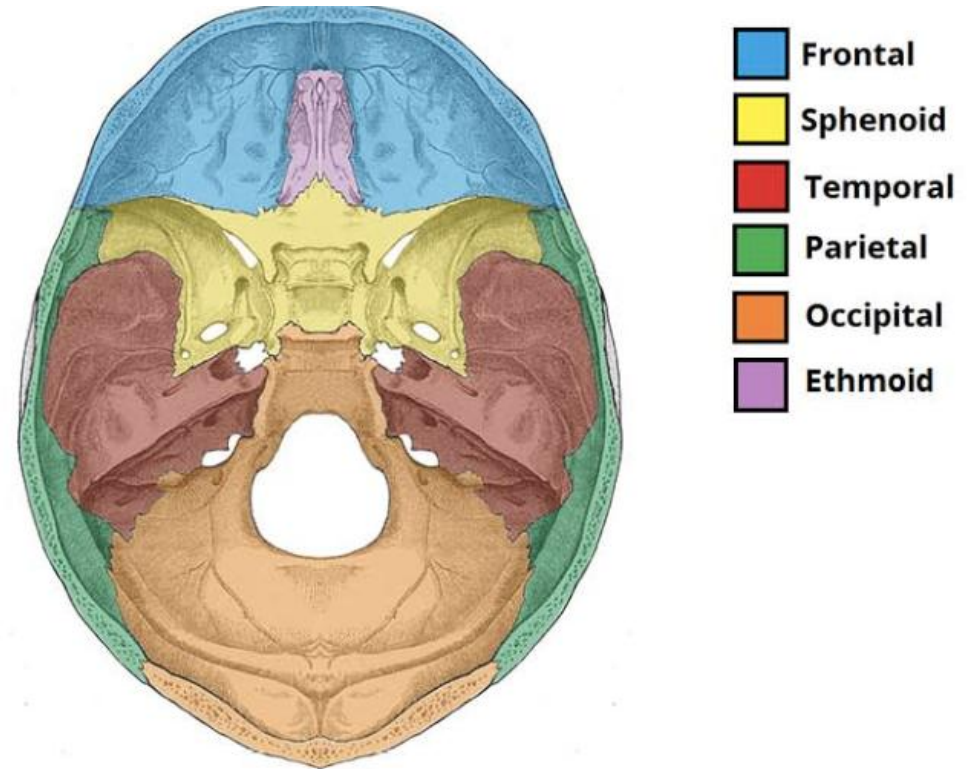
LEE ANN CUNY, DO

Objectives – Base (Sphenoid and Temporal)

- ▶ Identify the individual bones of the cranial base, their developmental parts and origin (cartilage or membrane) and locations in the adult osteology
- ▶ Describe the physiologic motion of the individual bones with the primary respiratory mechanism (PRM) axis of motion
- ▶ Identify patient presentations that would indicate a need to evaluate and possibly treat the bones of the cranial base, including possible cranial nerve impingements
- ▶ List the foramen present in the bones of the cranial base and the relevant structures that traverse.

Cranial Base

- ▶ Bone formed in cartilage secondary to compression
 - ▶ Ethmoid
 - ▶ Basi-Sphenoid, medial parts of greater and lesser wings
 - ▶ Basi-occiput, condylar parts, supraocciput
 - ▶ Temporal x 2
 - ▶ Petrous portion
 - ▶ Mastoid portion



Sphenoid

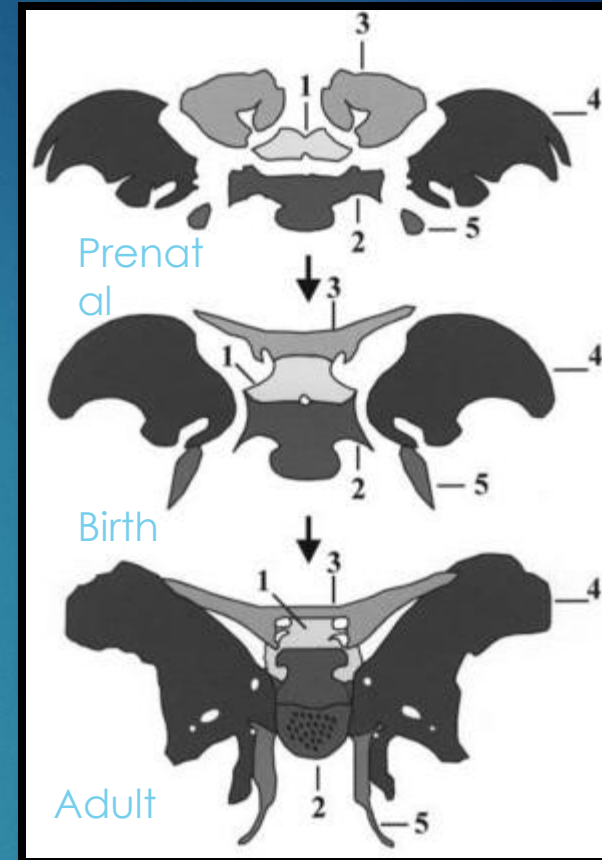


► Cartilaginous Ossification

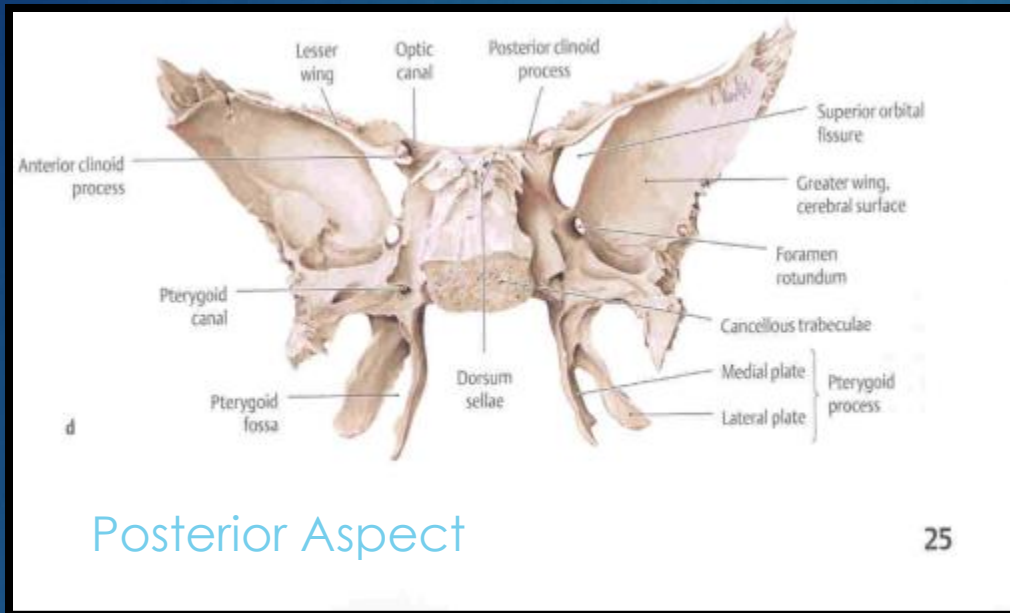
- Sphenoid Body- Develops in 2 parts
 - Presphenoid
 - Tuberculum Sellae with lesser wings
 - 2 ossification centers of the body 1 for each wing
 - Postsphenoid
 - Sella Tursica, Dorsum Sellae
 - 2 ossification centers for the body portion, 1 for each lingula
- The Lesser Wings
- Proximal aspects of the greater wings and pterygoid plates

► Membranous Ossification

- Distal aspects of the greater wings and pterygoid plates

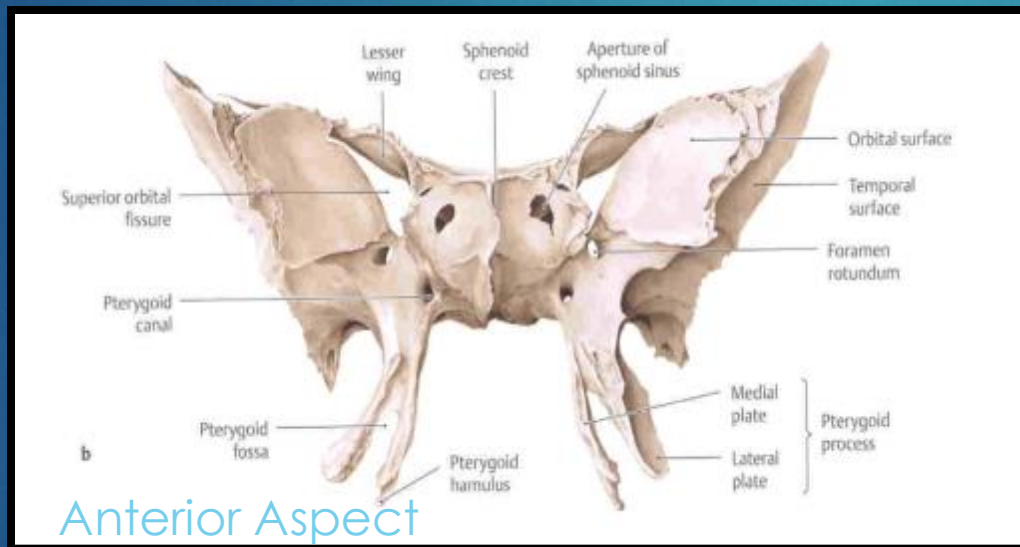


Embryologic Origin/Fetal Vs Adult Sphenoid

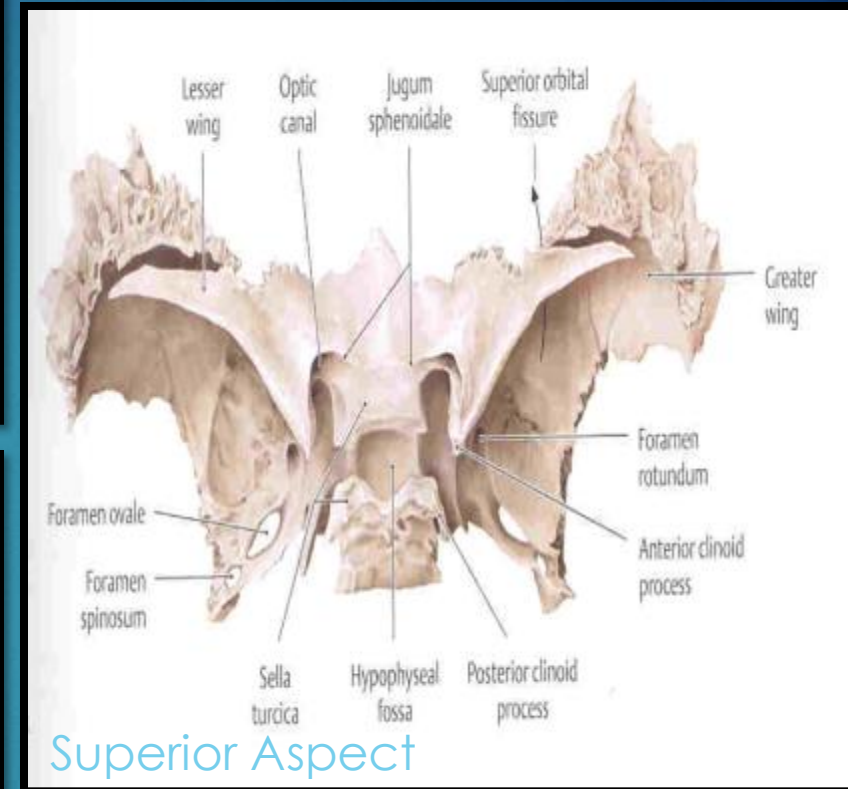


Posterior Aspect

25



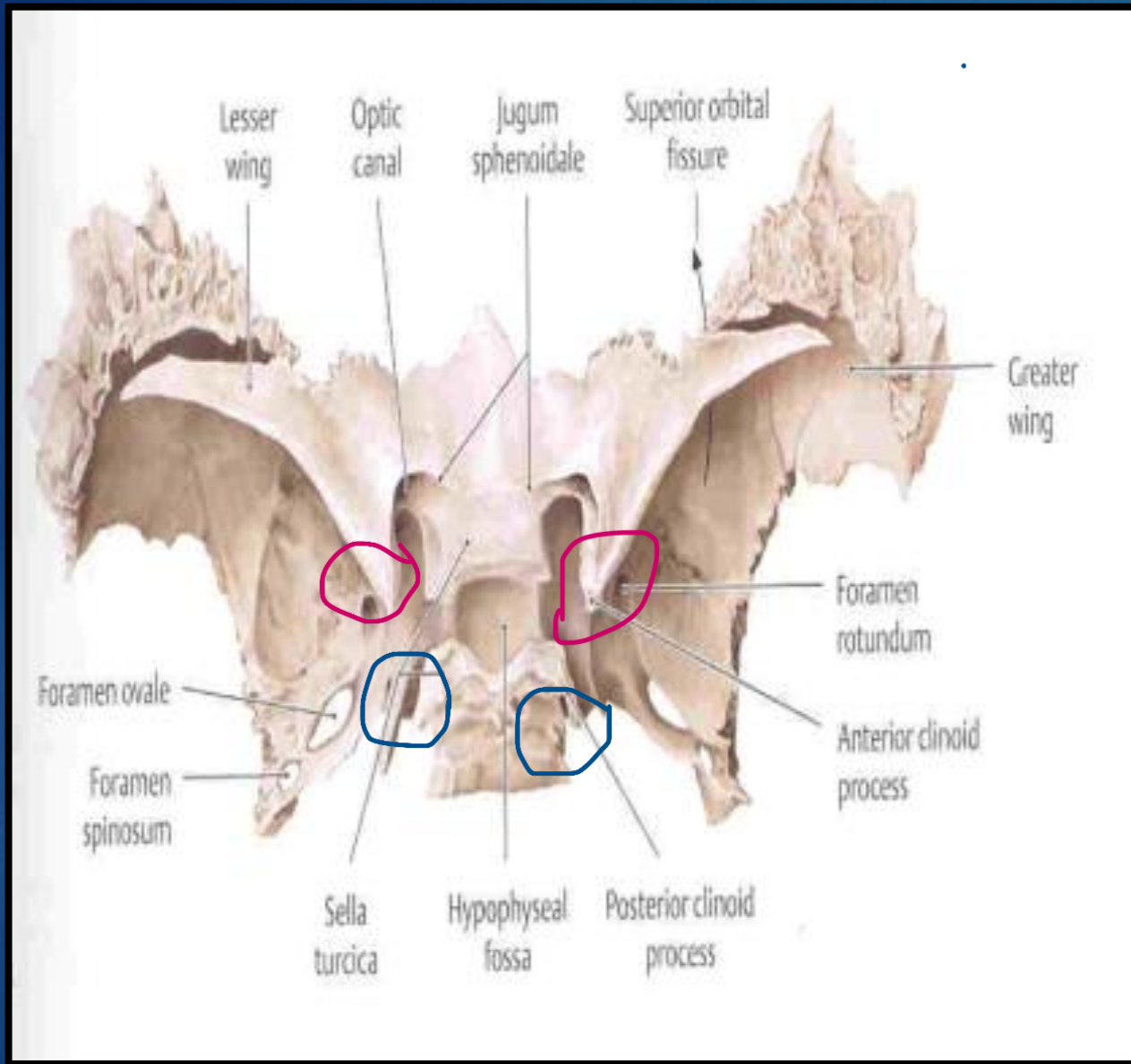
Anterior Aspect



Superior Aspect

greater wings, lesser wings, optic canal, superior orbital fissure, ant post clinoid processes, hypophyseal fossa, lateral medial plat of pterygoid fossa

Parts of the Sphenoid



► Tentorium Cerebelli

- Free border
 - Attaches to the anterior clinoid process
- Fixed border
 - Attaches to the posterior clinoid process

Attachment of the Falx Cerebelli

Identification of the Sphenoid Parts & Articulations

Parts of the Sphenoid

- Body-Sella Turcica
- Two lesser wings
- Two greater wings
- Two pterygoid processes

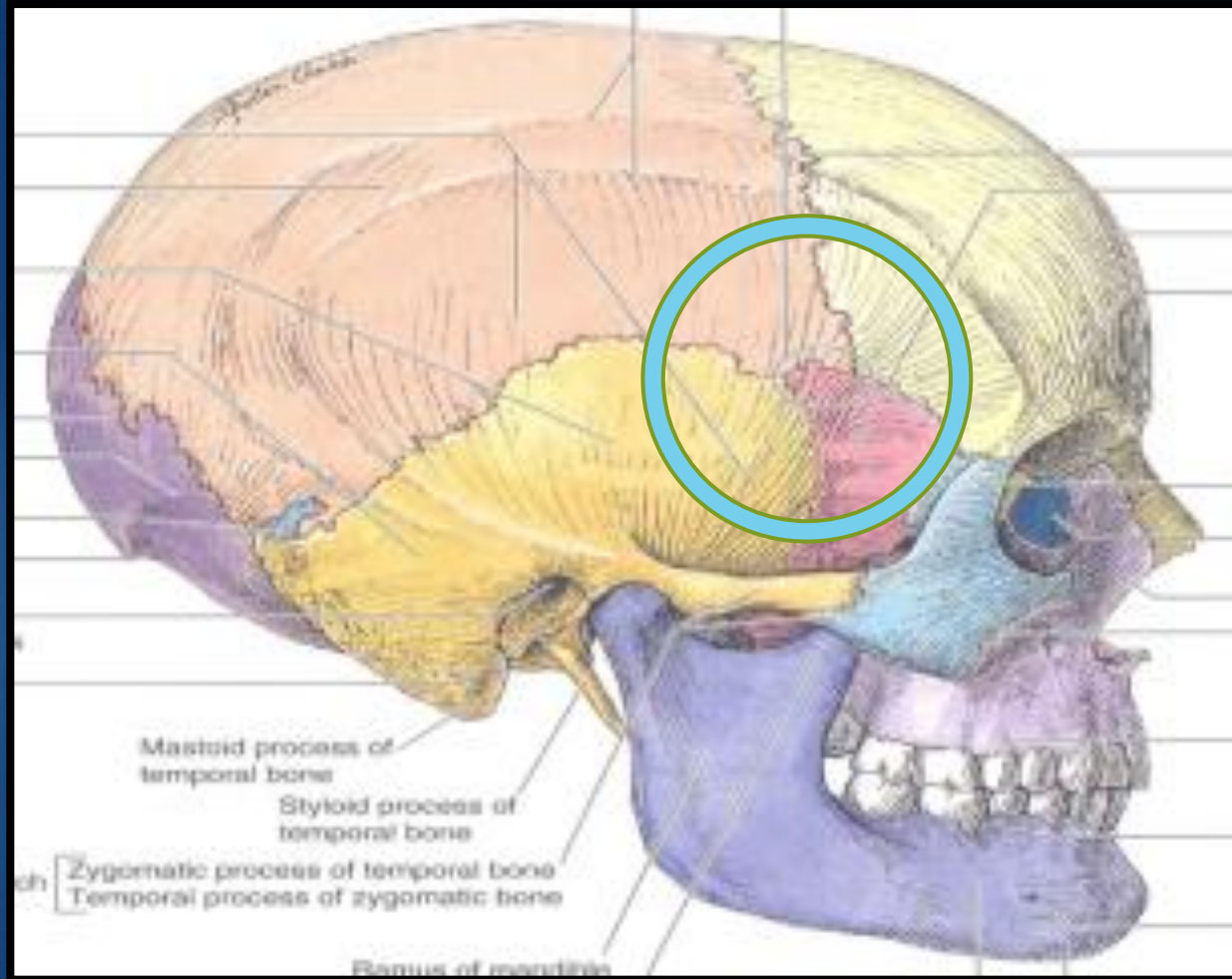
Articulates with:

1. Occipital
2. Temporal
3. Parietal
4. Frontal
5. Zygomae
6. Palatine
7. Vomer
8. Ethmoid

be
aware
of the
articulations



Identification of Sphenoid Parts & Articulations

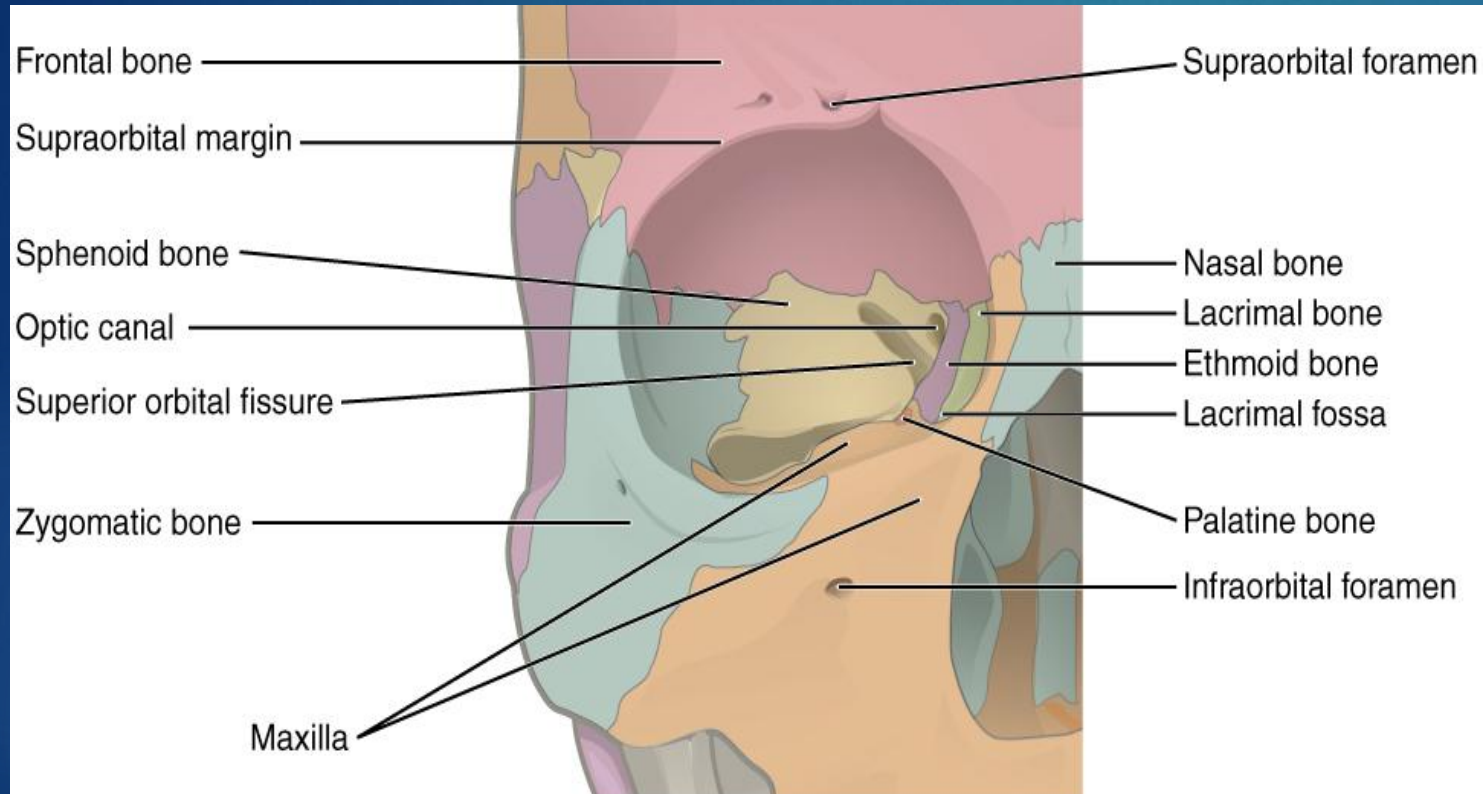


Articulations at Pterion

- Temporal
- Parietal
- Frontal
- Sphenoid

Superficial → Deep
Temporal, Sphenoid, Parietal, Frontal

Identification of Sphenoid Parts & Articulations



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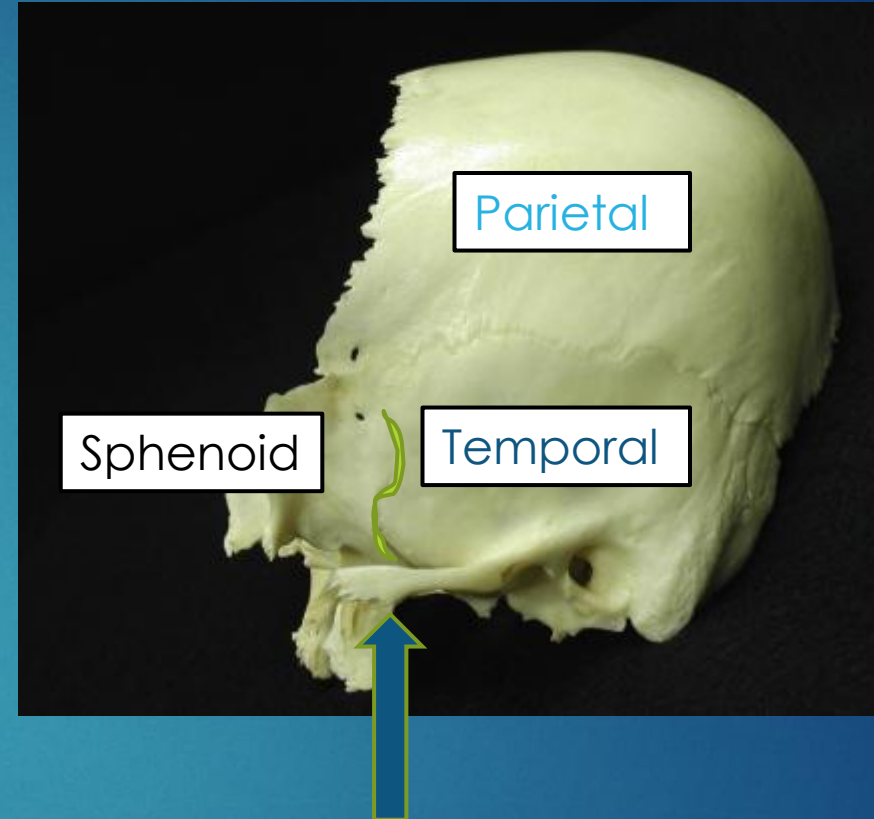
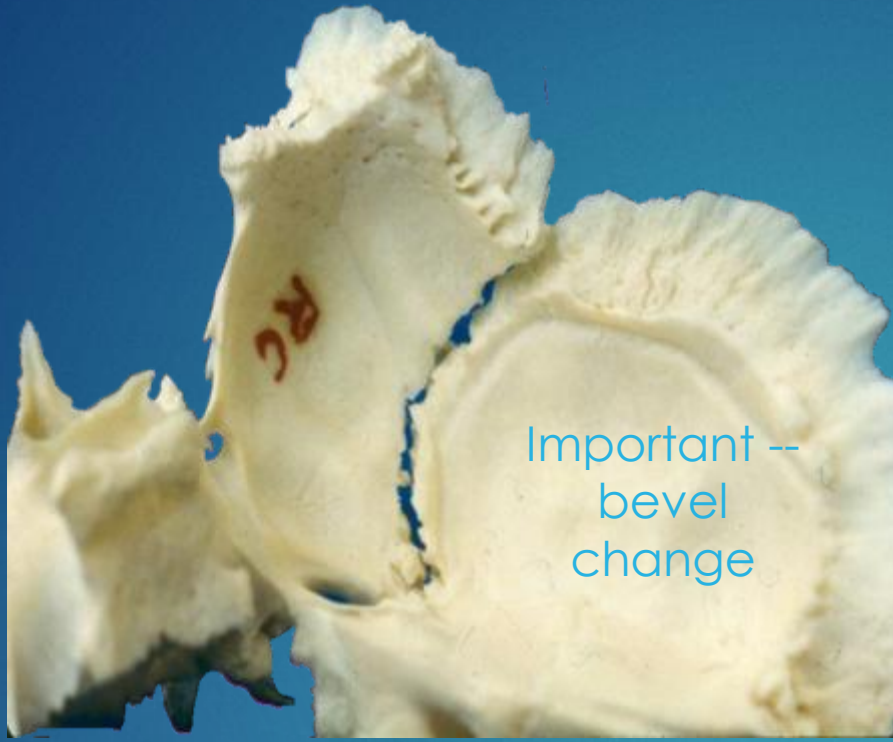
Frontal (red)
Maxilla (orange)
Sphenoid (yellow)
Zygoma (blue-green)
Lacrimal (lime green)
Palatine (barely visible red-orange)
Ethmoid (purple)



Sphenobasilar Synchondrosis (SBS)

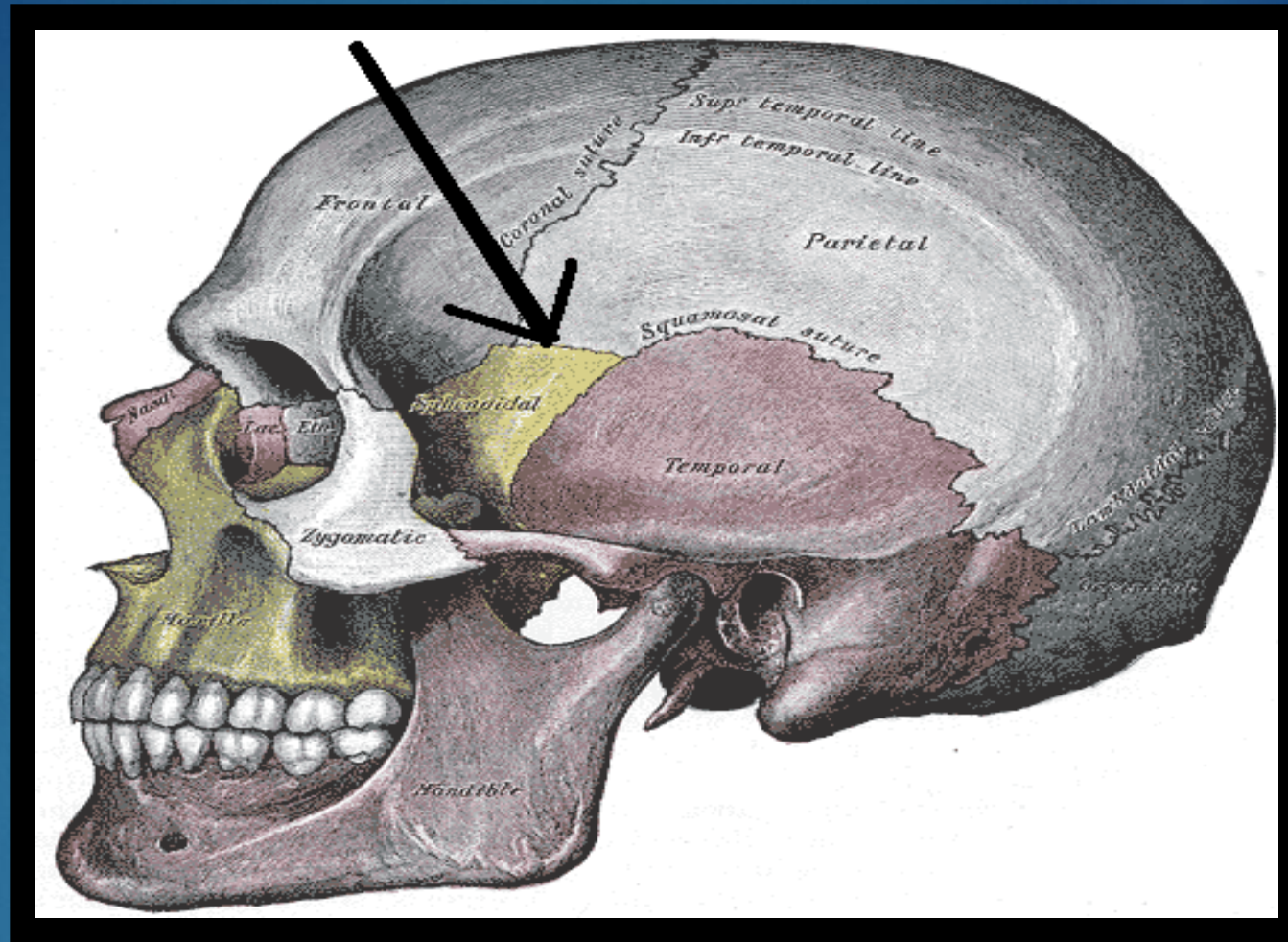
Between body of sphenoid and basilar portion of occiput

normal motion and f/e in vault



Sphenosquamous Articulation

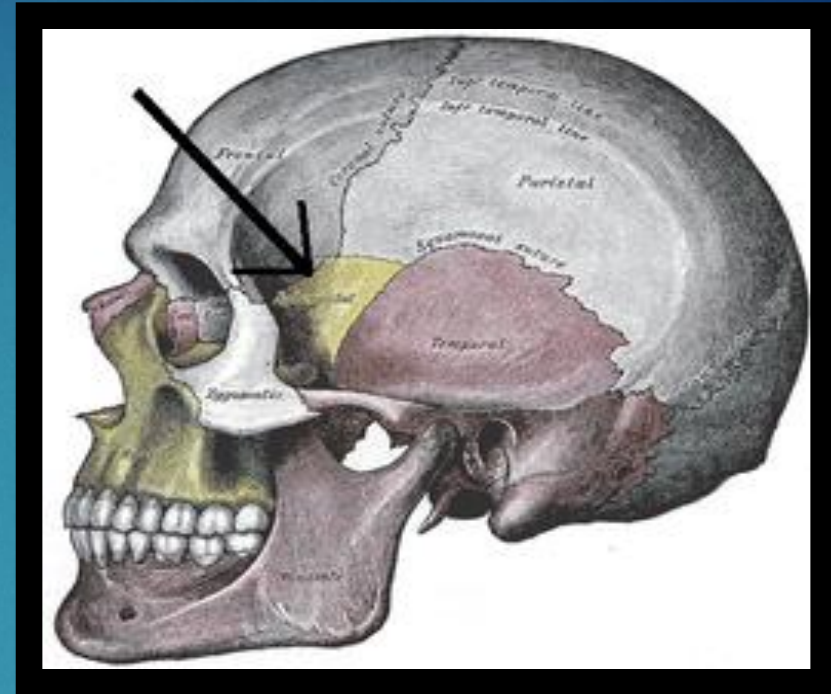
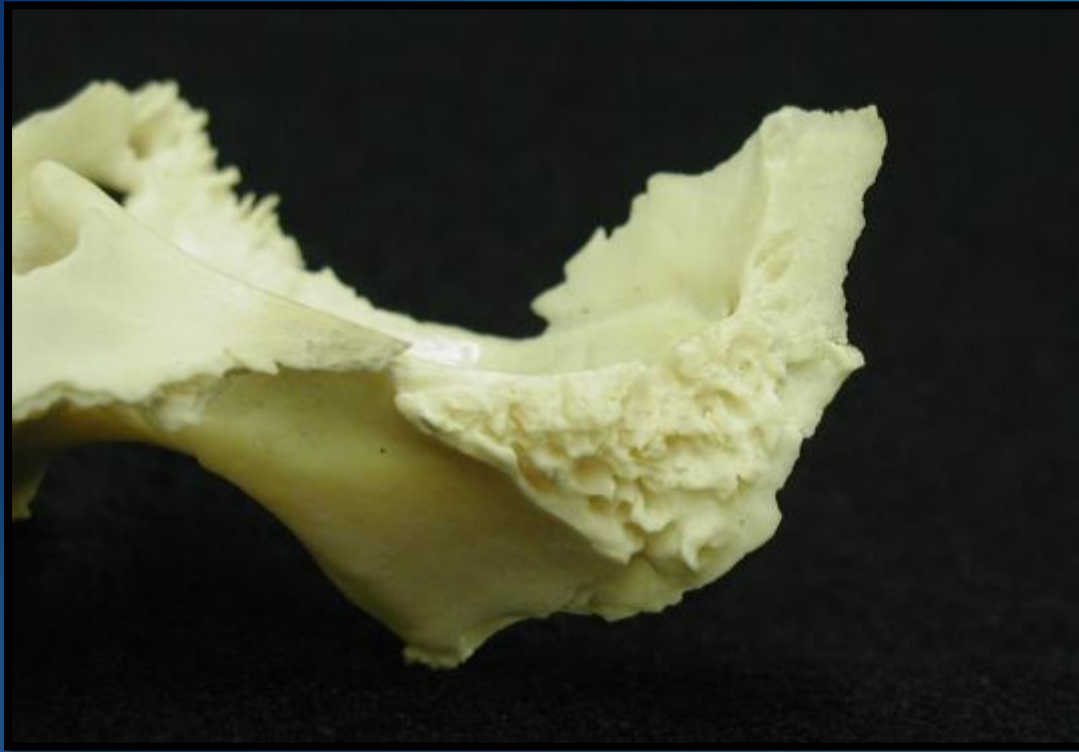
ss pivot Between sphenoid and at squamous temporal
 ** bevel change at the level of the zygomatic process



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Sphenoparietal Suture

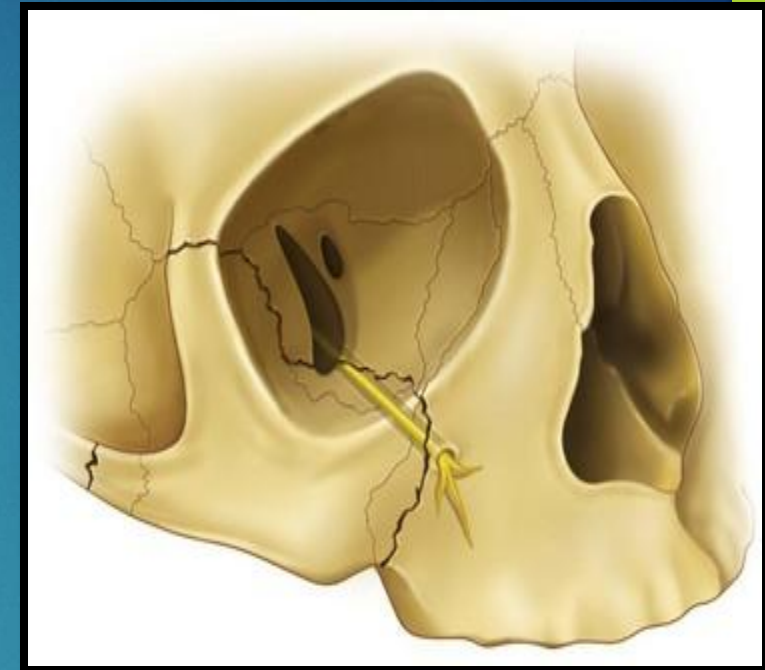
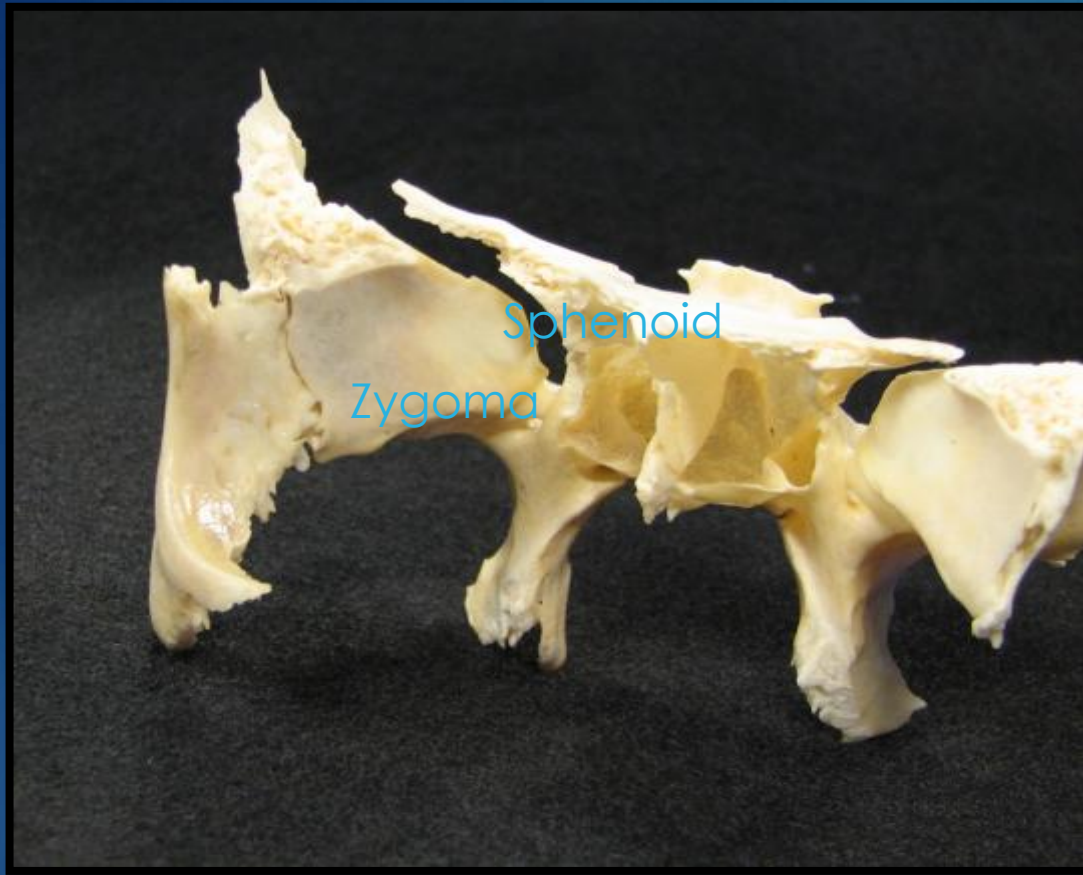
Articulation with the parietal bone



Frontosphenoidal Articulation

triangular shaped

Articulation with the frontal bone

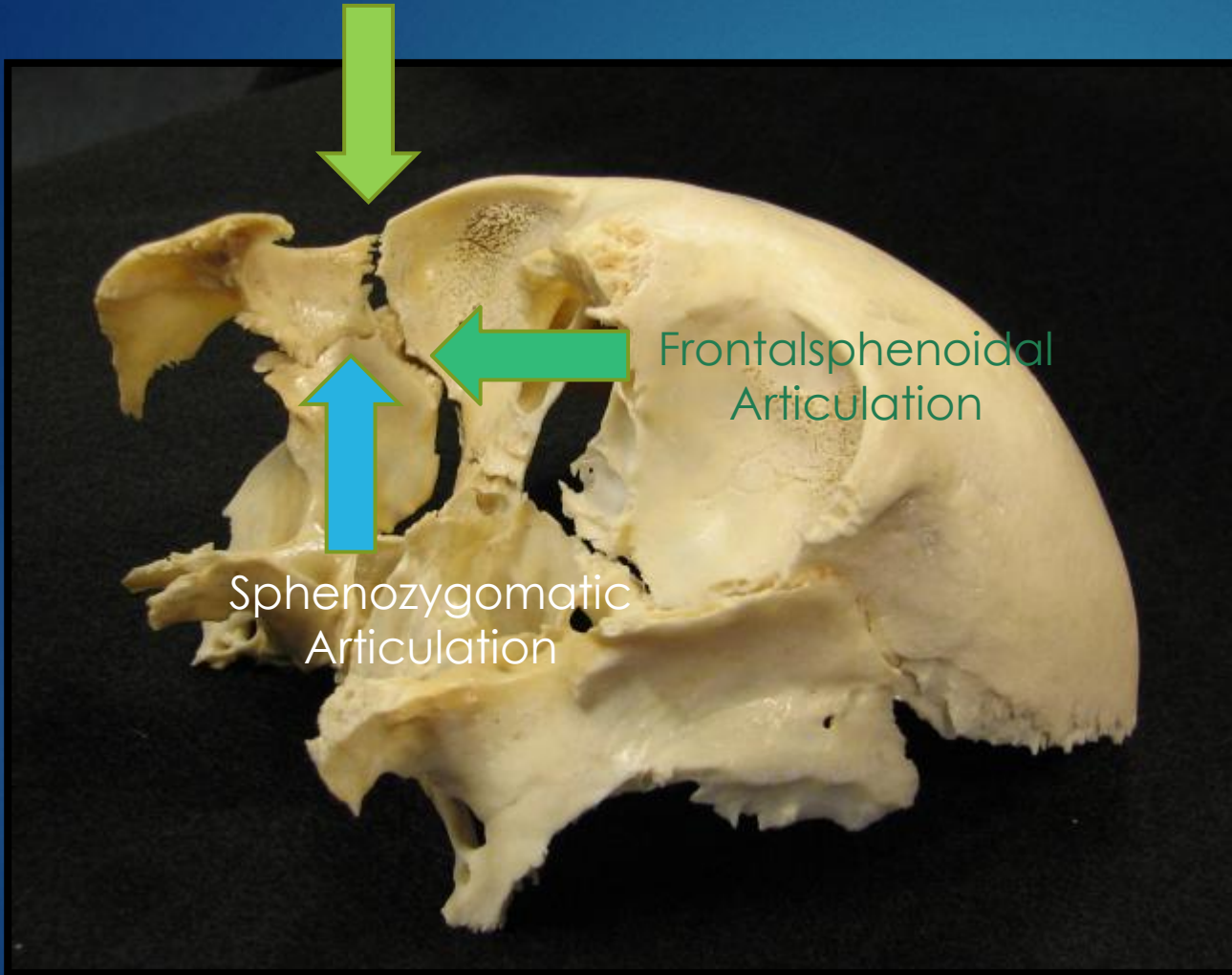


<https://pocketdentistry.com/16-fractures-of-the-zygomatic-complex-and-arch/>

Sphenozygomatic Articulation

Articulation with the Zygoma

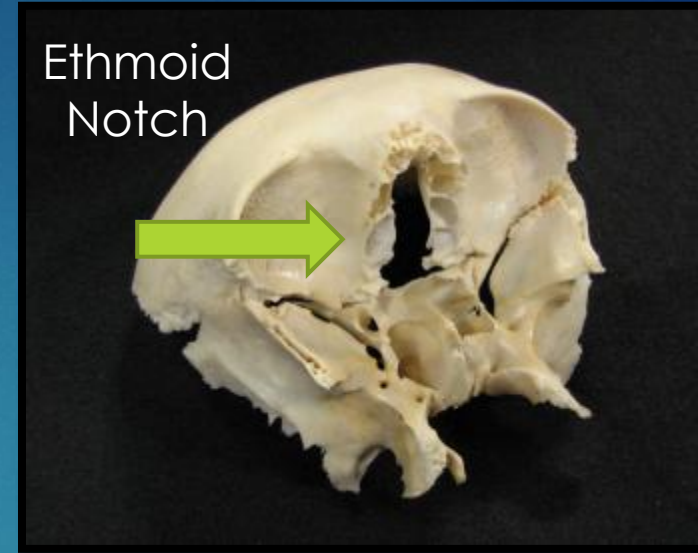
Frontozygomatic
Articulation



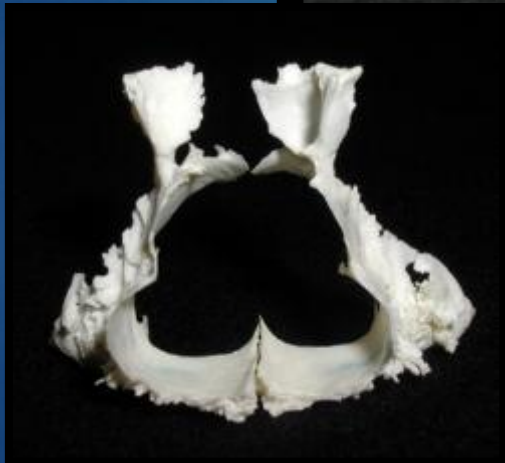
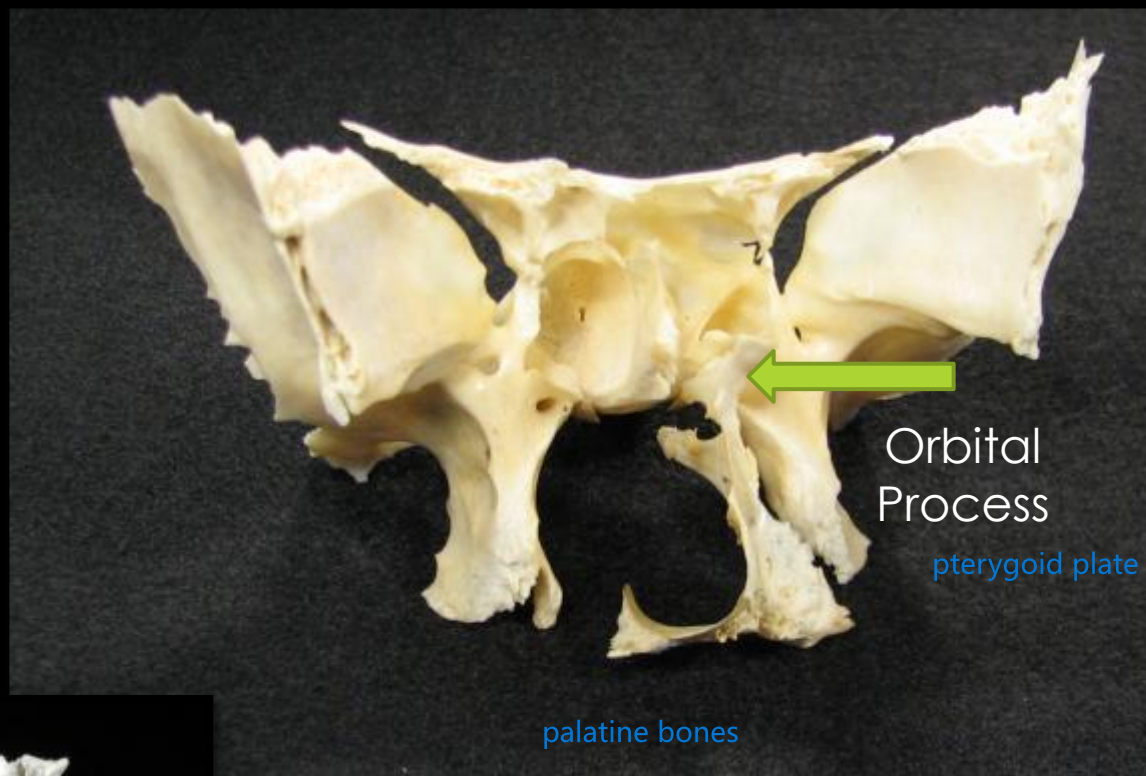
Frontalsphenoidal
Articulation

Sphenozygomatic
Articulation

Ethmoid
Notch

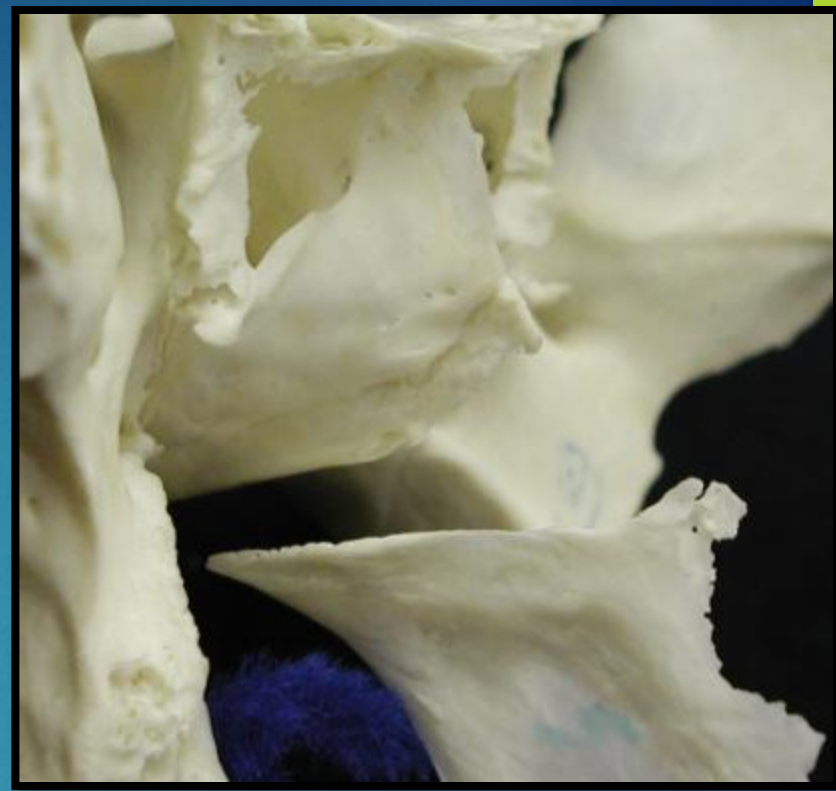
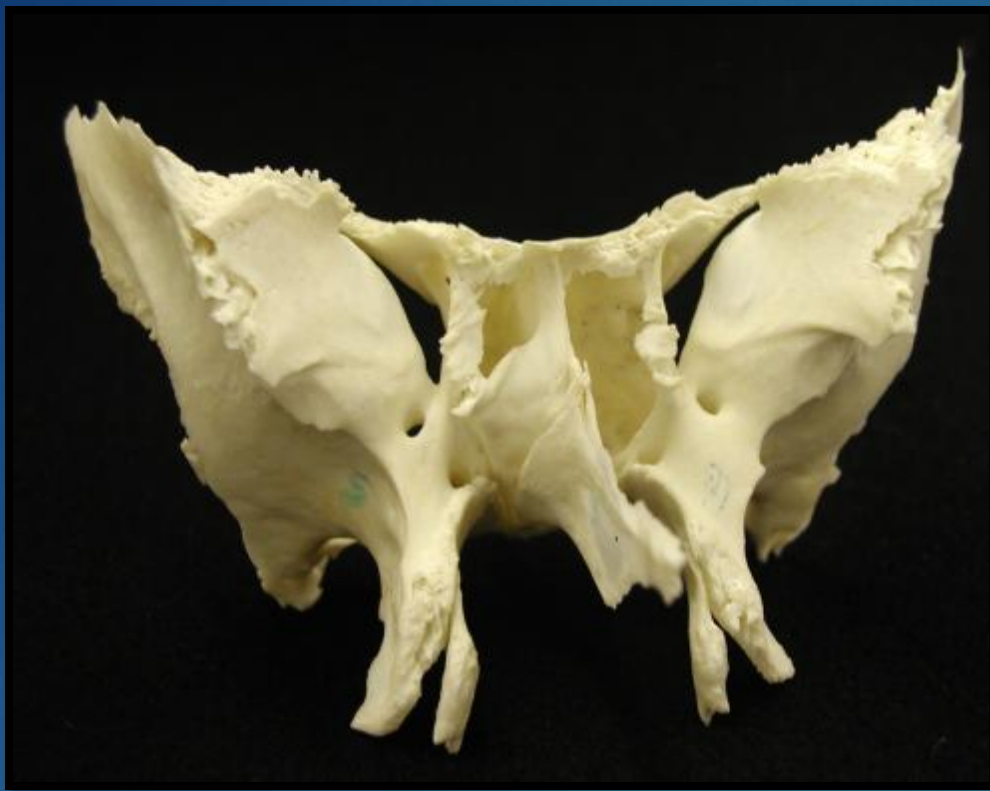


Putting the Articulations Together

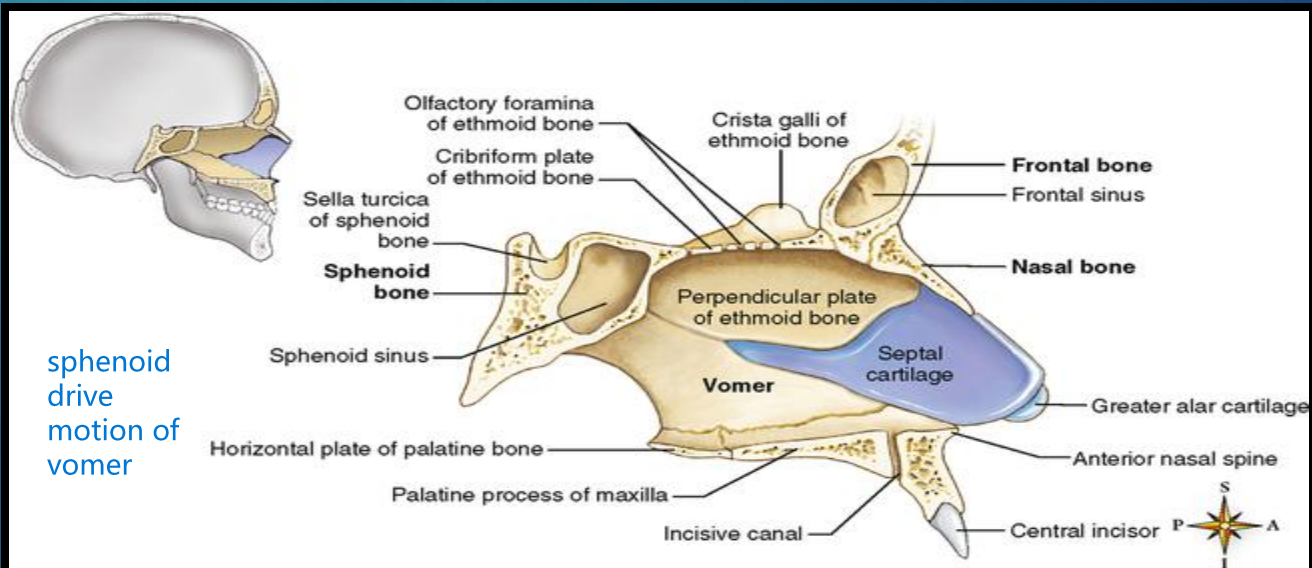


Sphenoid and Palatines

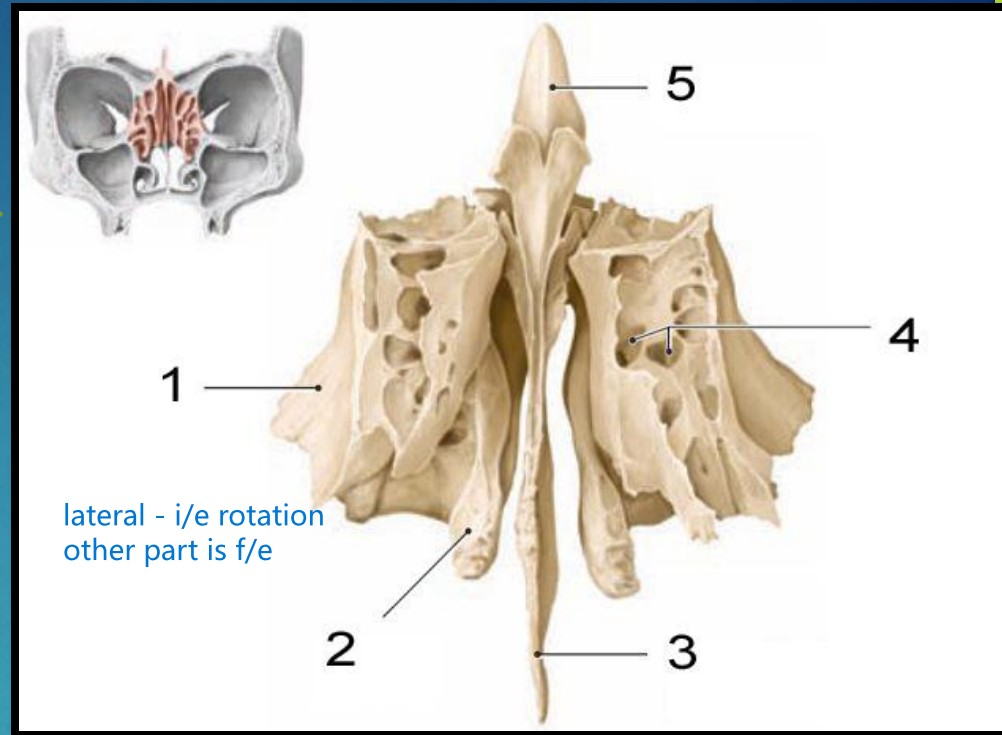
Articulation with the palatine bones



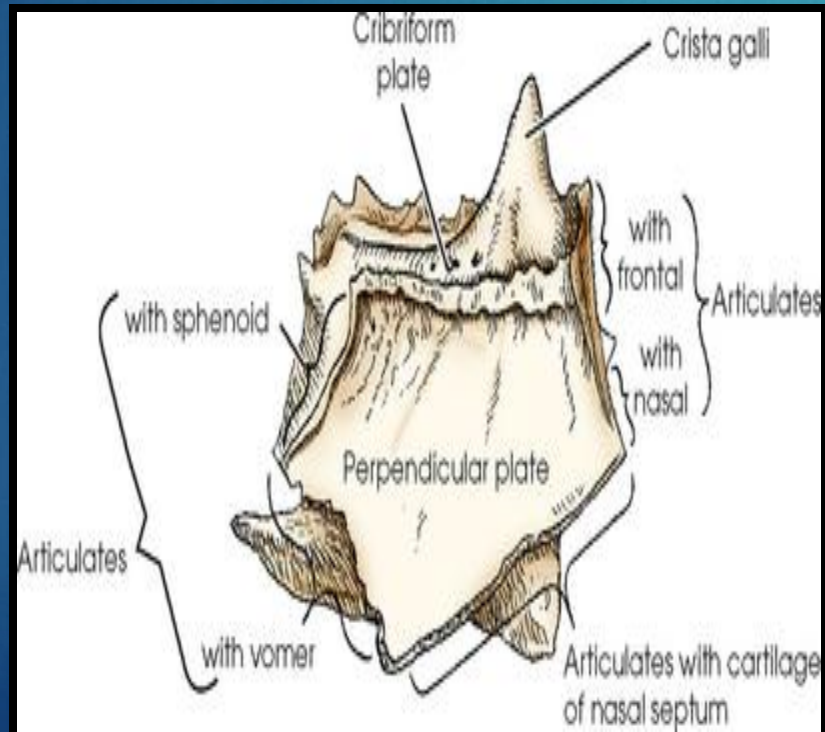
Sphenoid-Vomer Articulation



1. Orbital plate
2. Middle Concha
3. Perpendicular Plate
4. Ethmoid Air Cells
5. Crista Galli



Posterior View Ethmoid

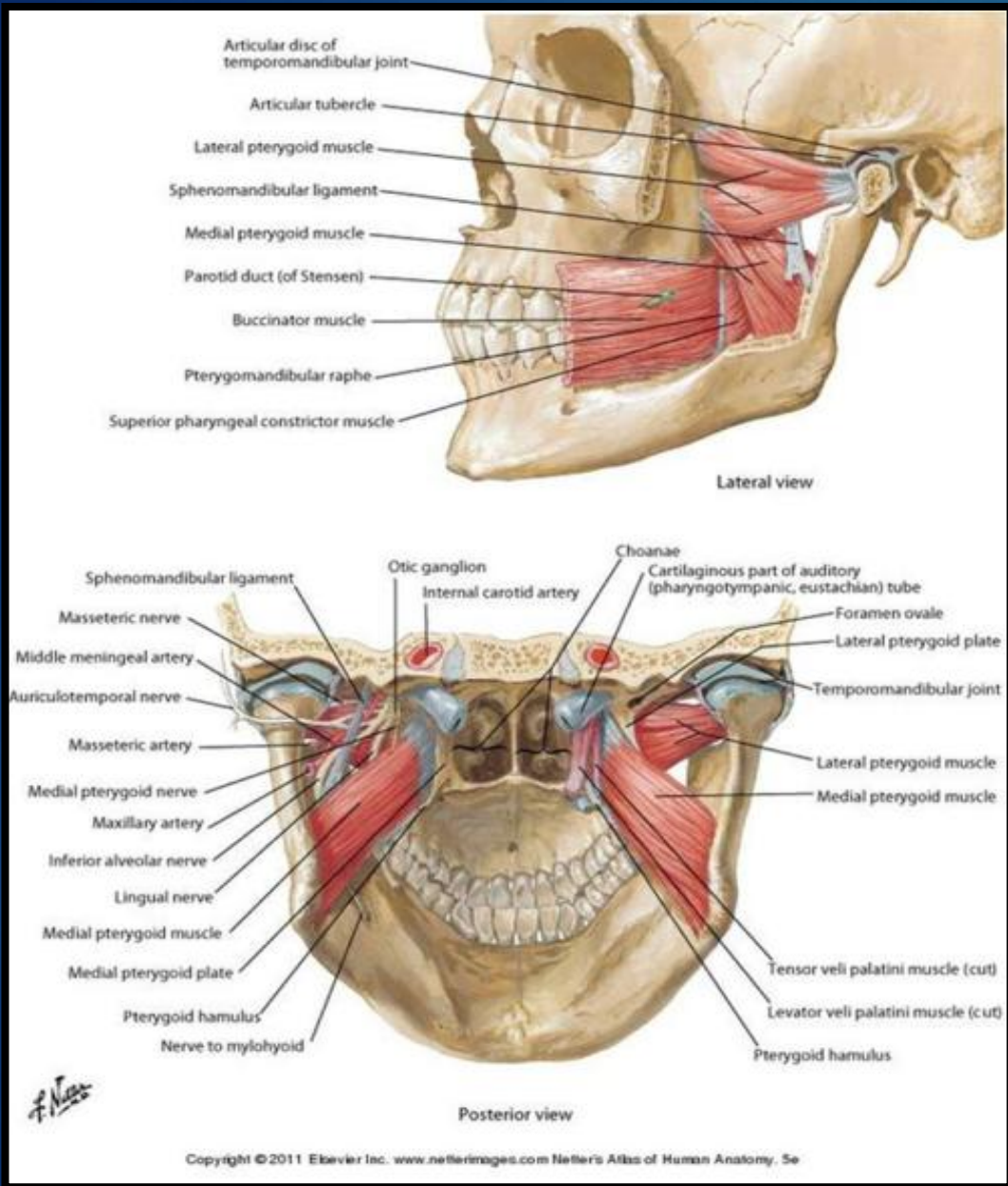


Lateral View Ethmoid

Spheno-Ethmoid Articulation



Important Muscular &
Ligamentous Attachments to
the Sphenoid



► The Pterygoid Processes

- Separated by a fissure which articulates with the palatine bone
- Diverge posteriorly creating a fossa
- Attachment point for:

1. Lateral Pterygoid

- Origin- greater wing of sphenoid and lateral pterygoid plate
- Insertion- TMJ and mandible
- V3 innervation
- Jaw protrusion and closure

2. Medial Pterygoid

- Origin: medial surface of the lateral pterygoid plate and pterygoid fossa
- Insertion: mandibular angle
- V3 innervation
- Jaw closure

3. Tensor Veli Palatini

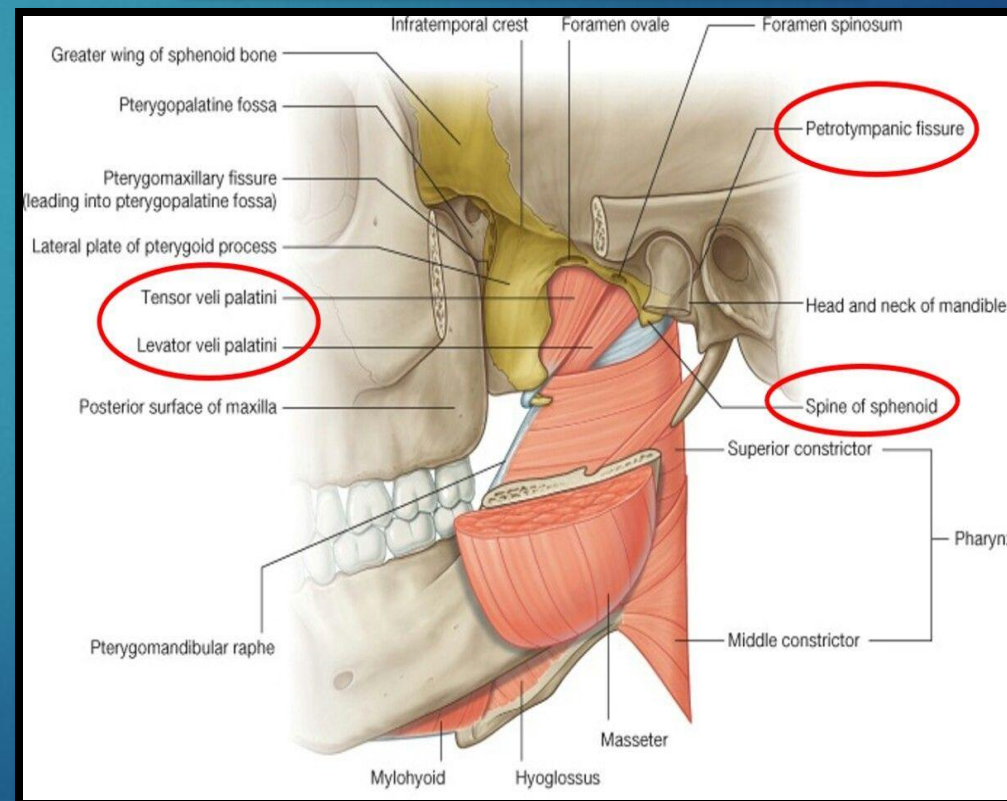
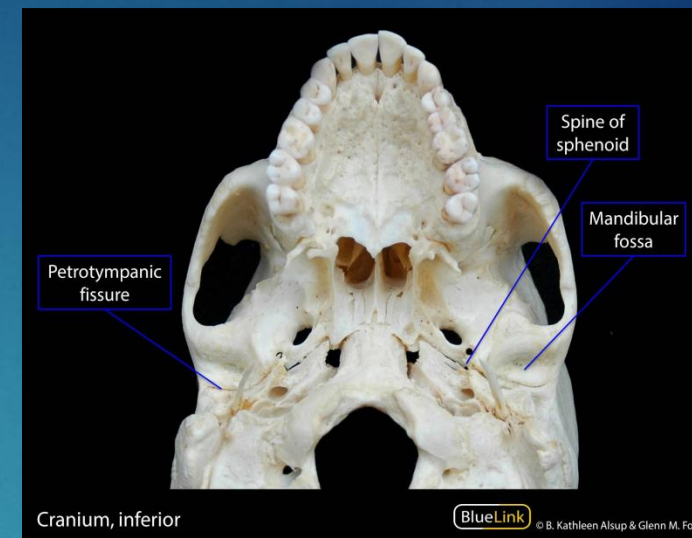
- Origin- medial pterygoid plate
- Insertion- palatine aponeurosis
- V3 innervation

Muscular Attachments at the Sphenoid

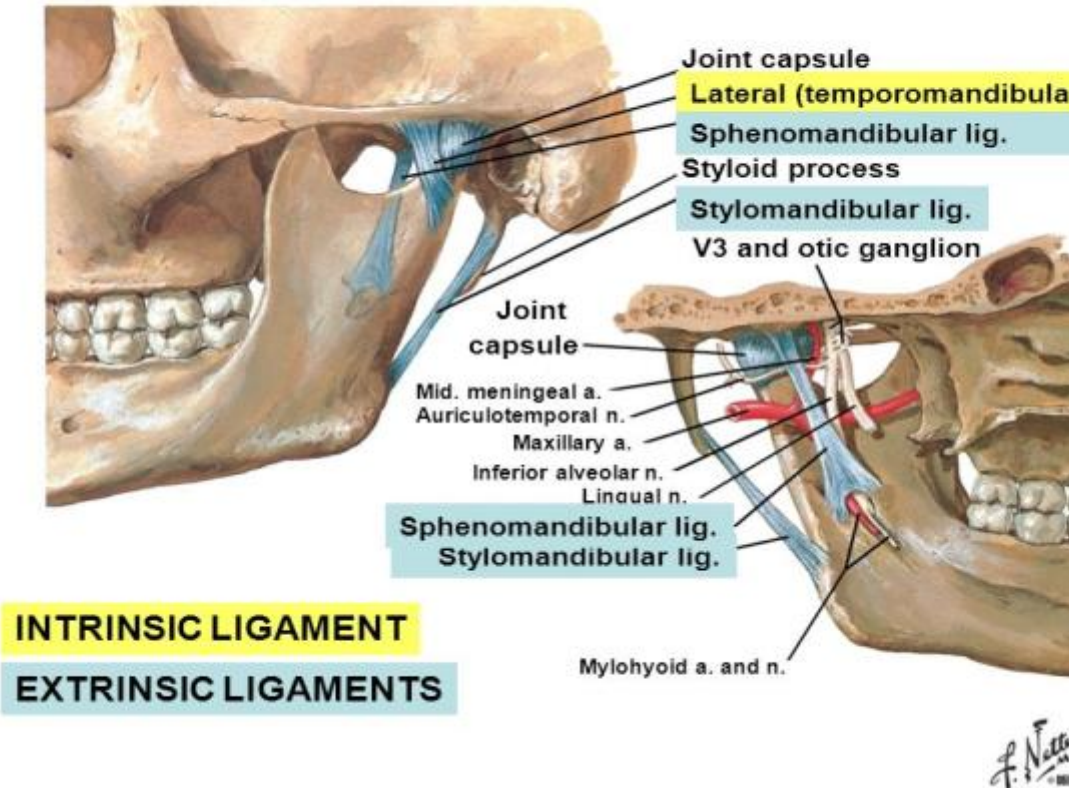
- Middle ear through the petrous portion of the temporal bone
- Across the articulation between the sphenoid and the petrous tip
 - The groove lies medial to the spine of the sphenoid
- Through pharyngeal musculature
 - Closely approximates tensor veli palatini and levator veli palatini and salpingopharyngeus
- Into the nasopharynx

Pharyngotympanic (eustachian) tube

can be impinged at any place during the passage

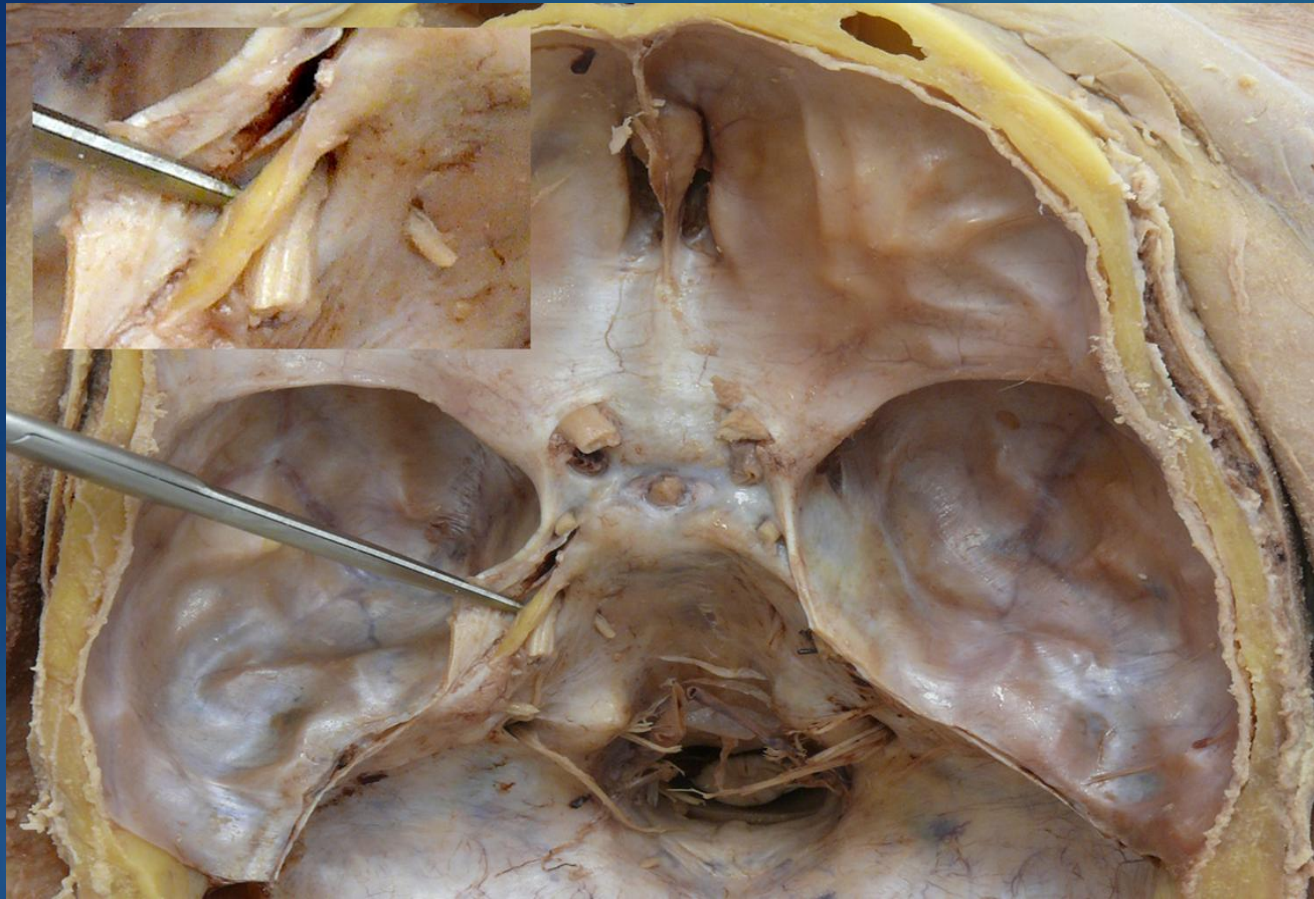


LIGAMENTS, NERVES & BLOOD SUPPLY OF TMJ



- Connects sphenoid to the lingula of the mandibular foramen
- Slackens when TMJ is closed, Taut when TMJ is open

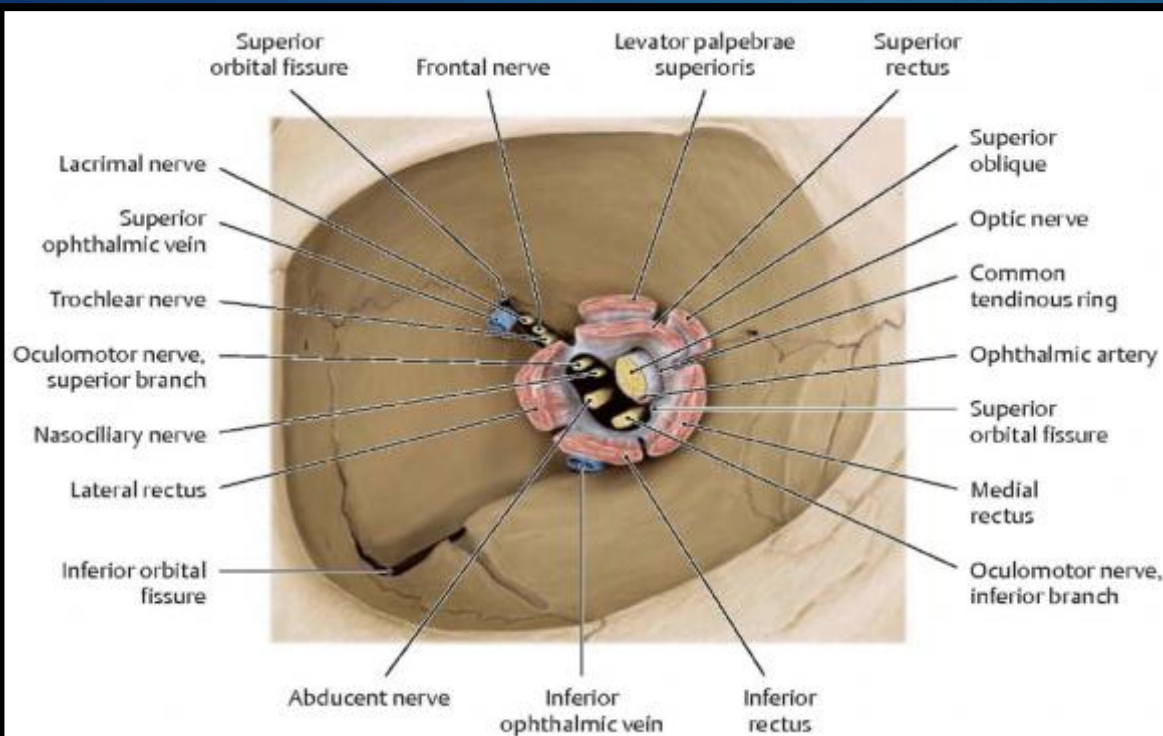
Sphenomandibular Ligament



under probe

Petrosphenoidal Ligament (Gruber's Ligament)

- From the petrous ridge of the temporal bone to the posterior clinoid process
- The Abducens Nerve Travels underneath it
- Tension here could effect contents of the cavernous sinus
- O TOM CAT
- O: oculomotor
- T: trochlear
- O: ophthalmic branch of V
- M: maxillary branch of V
- C: Internal Carotid Artery
- A: Abducent nerve
- T: Trochlear Nerve



Illustrator: Karl Wesker

pp. 138-139

Schuenke et al. THIEME Atlas of Anatomy • Head and Neuroanatomy

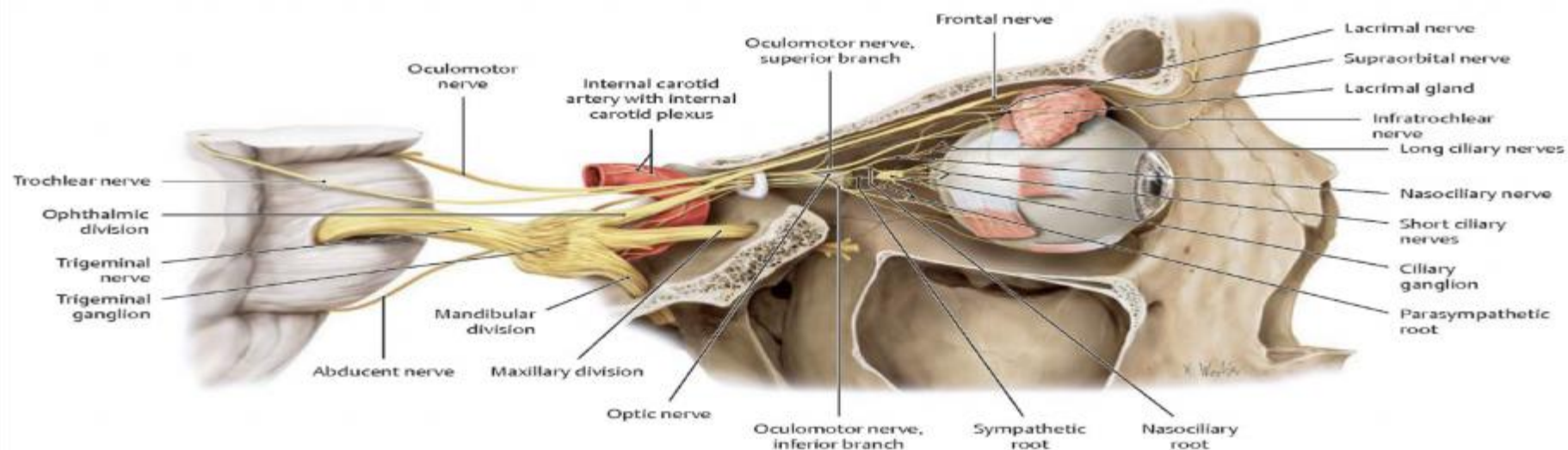
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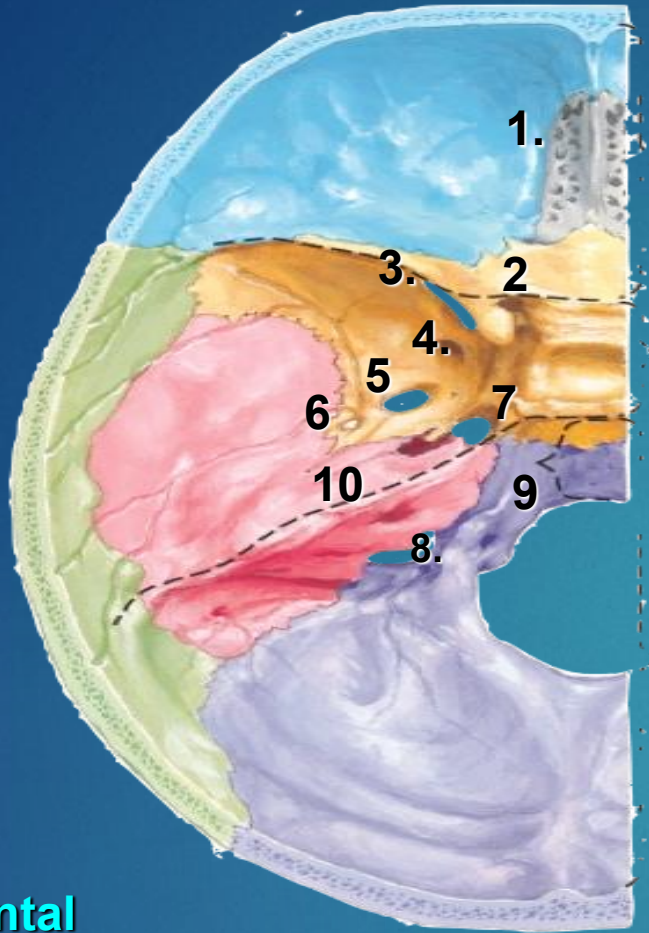


Common Tendinous Ring (annulus of Zinn)

- Point of attachment of the four rectus muscles
- Ophthalmic artery
- Sheath of CN II
- CN III, VI & V1 (nasociliary nerve)
 - Moore pg 902
- Crosses the optic canal and the superior orbital fissure and then attaches to the greater wing of the sphenoid
 - "It will be influenced by the mechanics of the lesser wing, greater wing and body of the sphenoid"

alteration in sphenoid can affect eye





Frontal

Ethmoid

Sphenoid

Temporal

Parietal

Occipital

1. cribriform plate

2. optic canal

3. superior orbital fissure

4. foramen rotundum

5. foramen ovale

6. foramen spinosum

7. foramen lacerum

8. jugular foramen

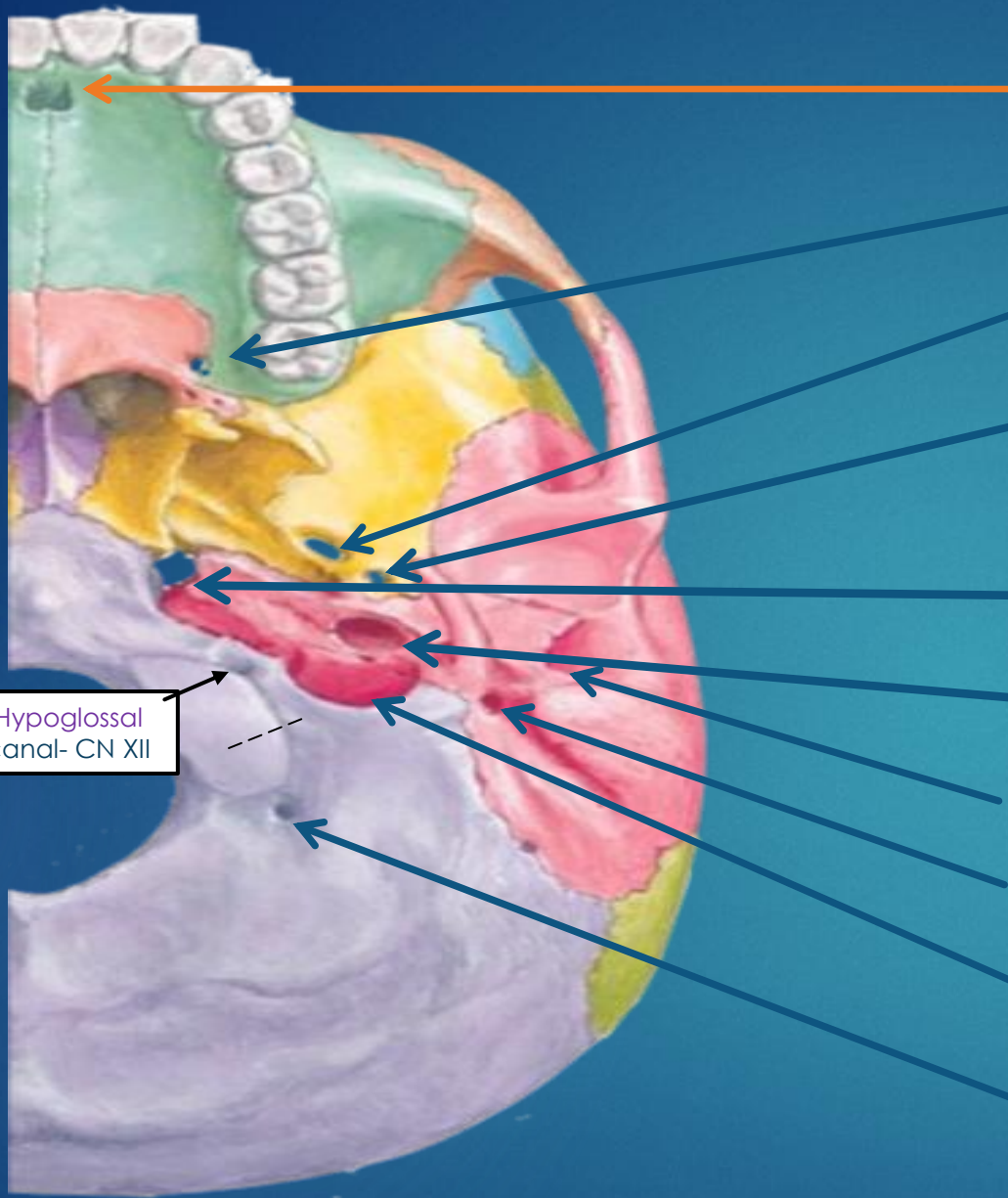
9. condylar canal

10. internal auditory meatus



Foramina

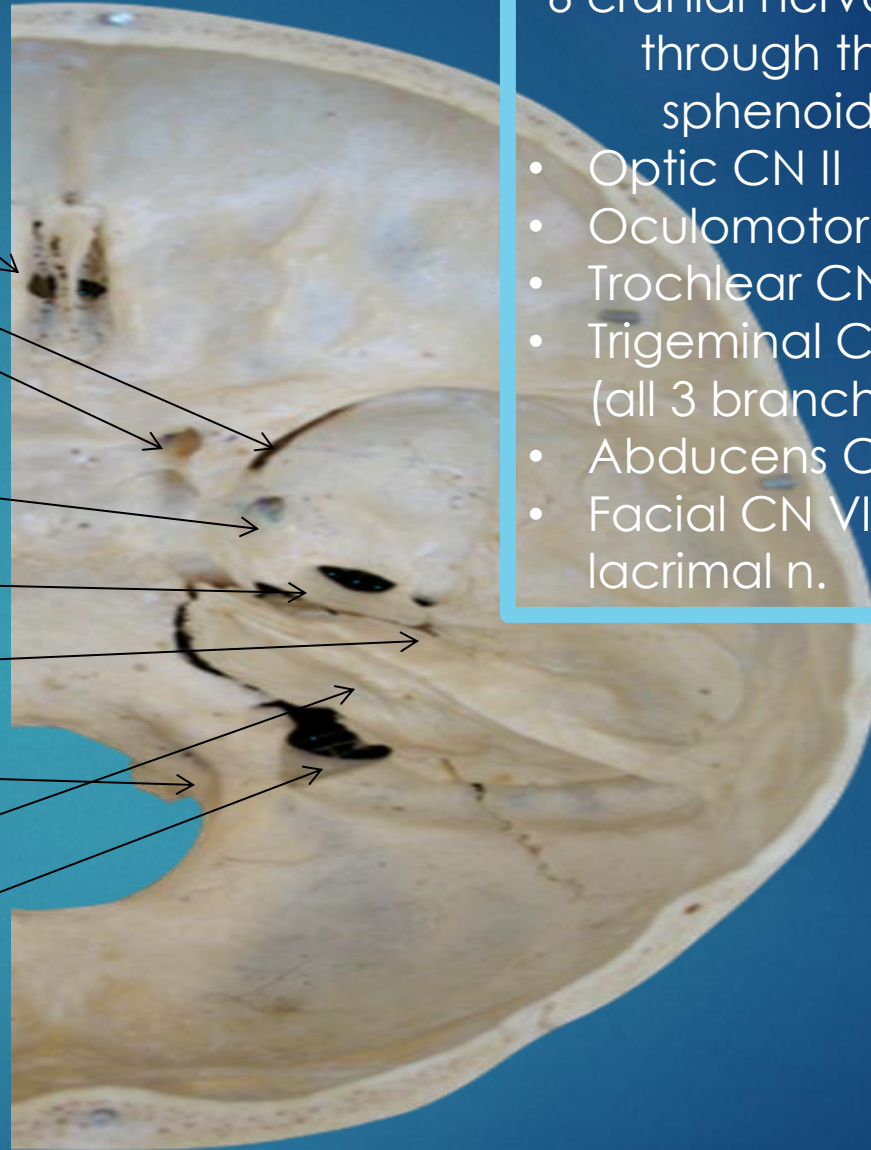
CN IX, X, XI through the jugular foramen



Hypoglossal
canal- CN XII

- Incisive Canal
 - Emissary vein
- Greater Palatine Foramen
 - CN V Maxillary- greater palatine n
- Foramen Ovale
 - CN V mandibular
- Foramen Spinosum
 - Middle Meningeal A.
- Foramen Lacerum
 - Symp fibers for deep petrosal nerve
- Carotid Canal
 - Internal Carotid Artery
- External Auditory Meatus
- Stylomastoid Foramen
 - CN VII motor branch
- Jugular Foramen
 - CN IX, X, XI
- Condylar Foramen
 - Emissary vein

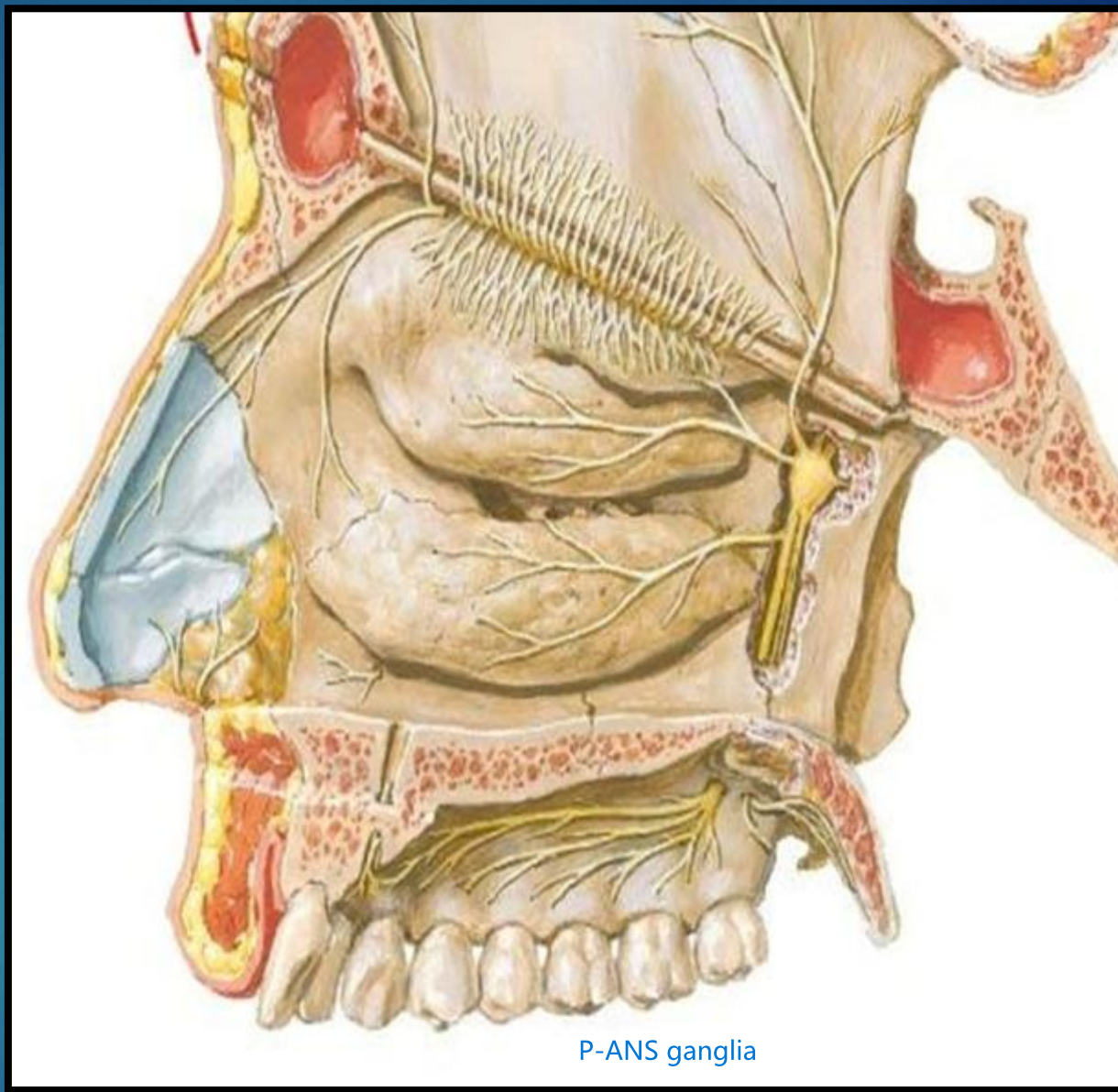
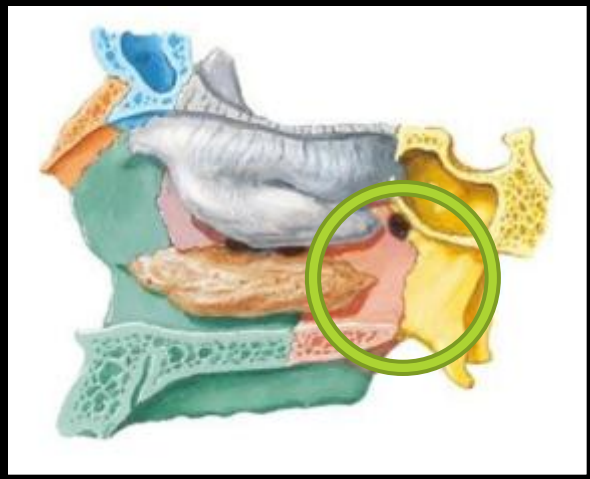
- ▶ Ethmoid Foramina
 - ▶ CNI- Olfactory
- ▶ Optic Canal
 - ▶ CNII- Optic
- ▶ Superior Orbital Fissure
 - ▶ CN III Ophthalmic N.
 - ▶ CN IV Trochlear N.
 - ▶ CN VI Abducens N.
 - ▶ CN V: ophthalmic N.
- ▶ Foramen Rotundum
 - ▶ CN V: maxillary division N.
- ▶ Foramen Ovale
 - ▶ CN V: mandibular division N.
- ▶ Foramen Spinosum
 - ▶ Middle meningeal artery
- ▶ Hypoglossal canal
 - ▶ CN XII: Hypoglossal N.
- ▶ Internal Auditory Meatus
 - ▶ VII: Facial N.
 - ▶ CN VIII: Vestibulocochlear
- ▶ Jugular Foramen
 - ▶ CN IX: Glossopharyngeal N.
 - ▶ CN X: Vagus N.
 - ▶ CN XI: Accessory N.



6 cranial nerves exit through the sphenoid:

- Optic CN II
- Oculomotor CN III
- Trochlear CN IV
- Trigeminal CN V (all 3 branches)
- Abducens CN VI
- Facial CN VII via lacrimal n.

Cranial Nerves that Traverse the Foramina, Internal Cranial View



The Pterygopalatine Fossa

posterior to teeth and superior



Clinical Indications for Evaluating/Treating Sphenoid

- ▶ Vision problems
- ▶ Strabismus
- ▶ Balance issues/vertigo
- ▶ Hearing problems
- ▶ URI/sinus congestion
- ▶ Sinus headache
- ▶ Bell's palsy

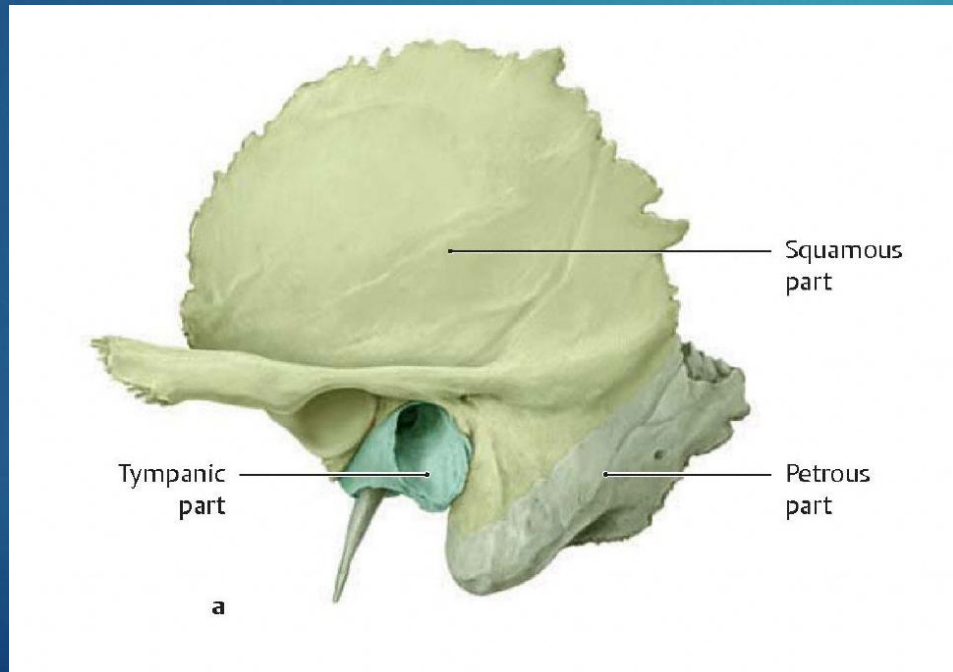
Temporal



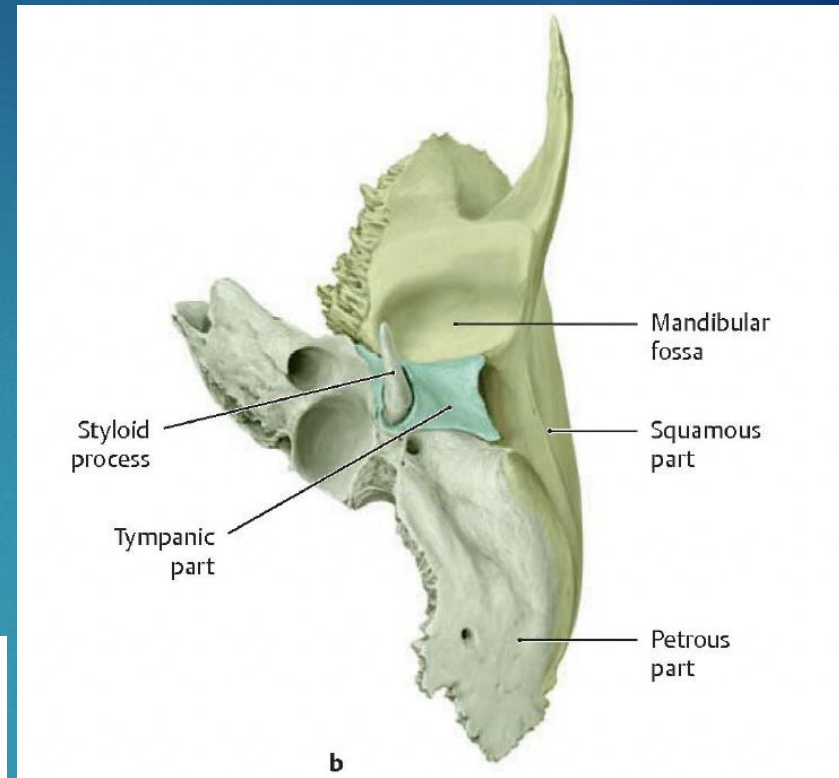
Origins

- ▶ Membranous origin
 - ▶ Squamous
 - ▶ Tympanic

Lateral View



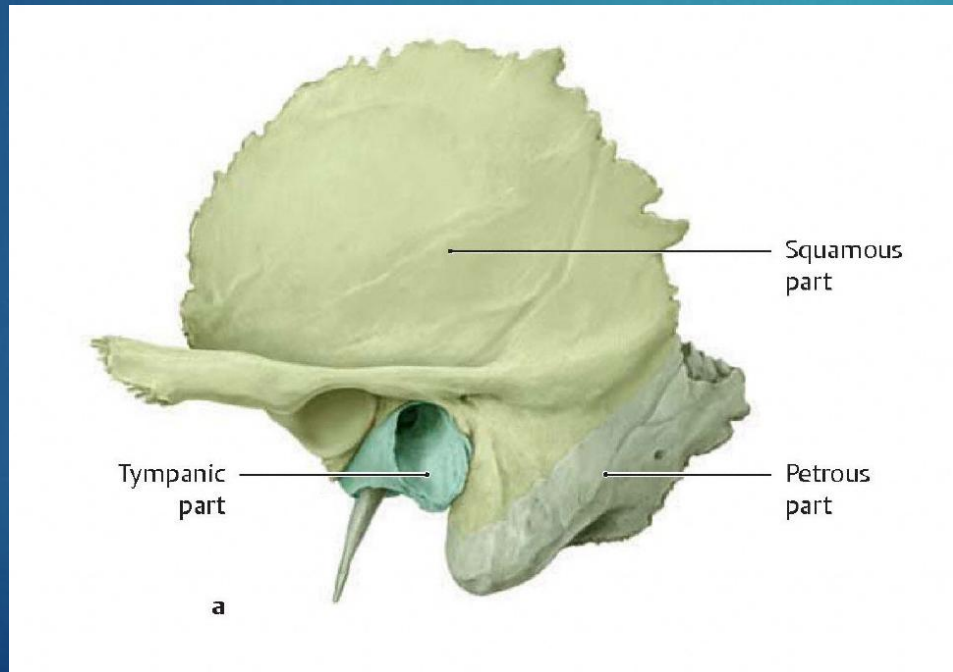
Inferior View



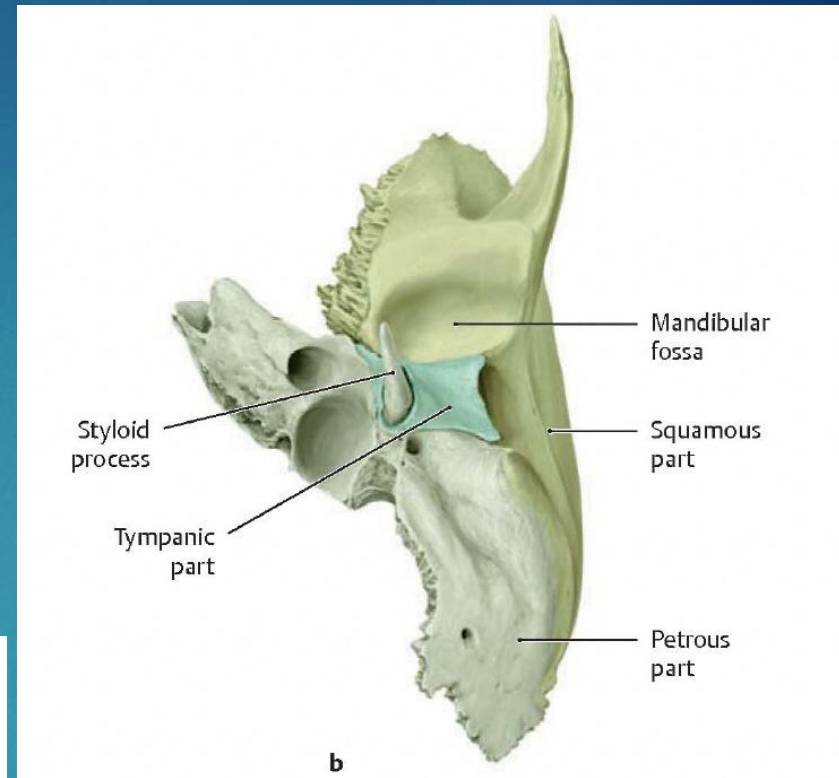
Origins

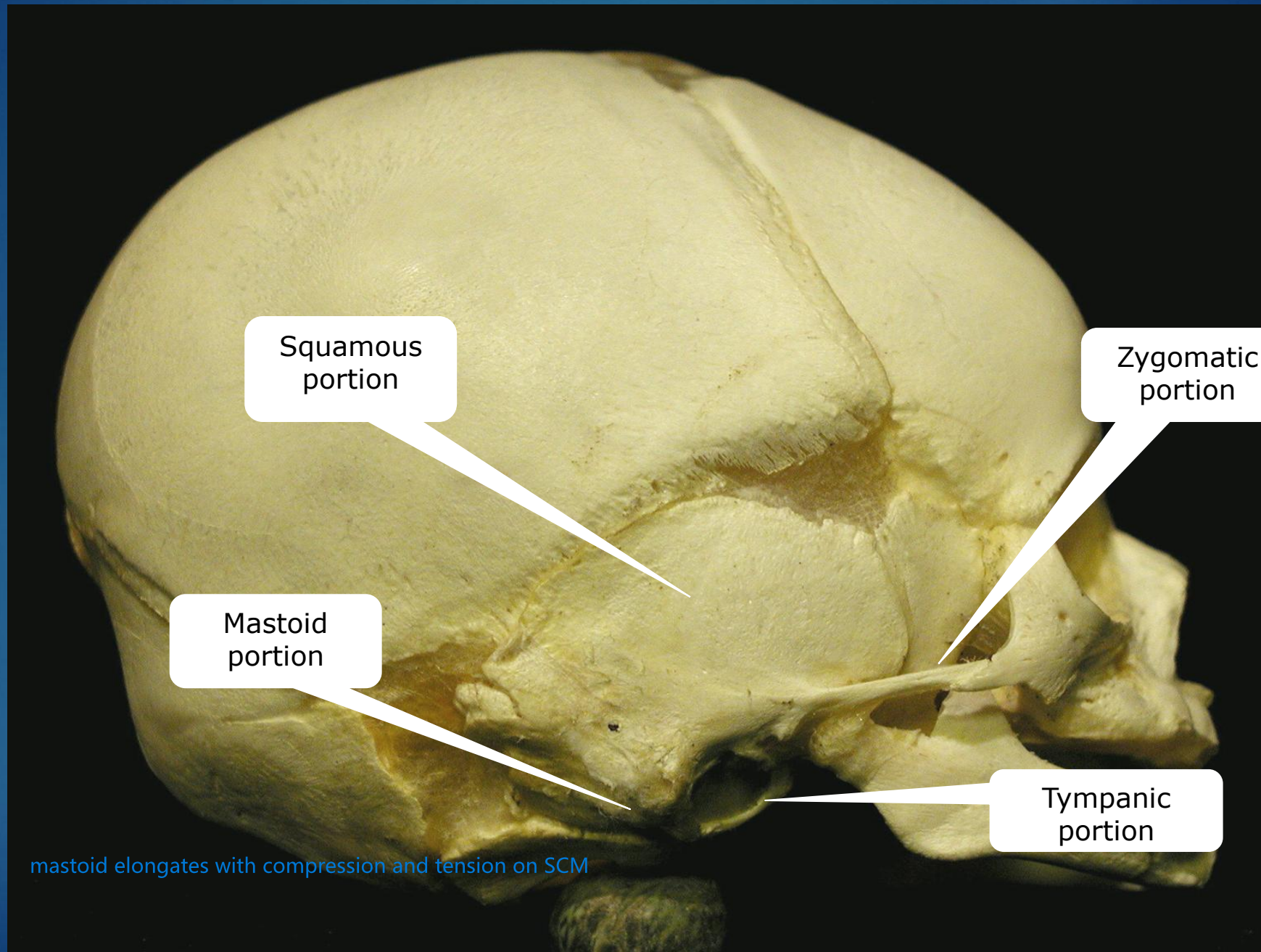
- ▶ Cartilaginous origin
 - ▶ Petrous
 - ▶ Mastoid
 - ▶ Styloid

Lateral View



Inferior View





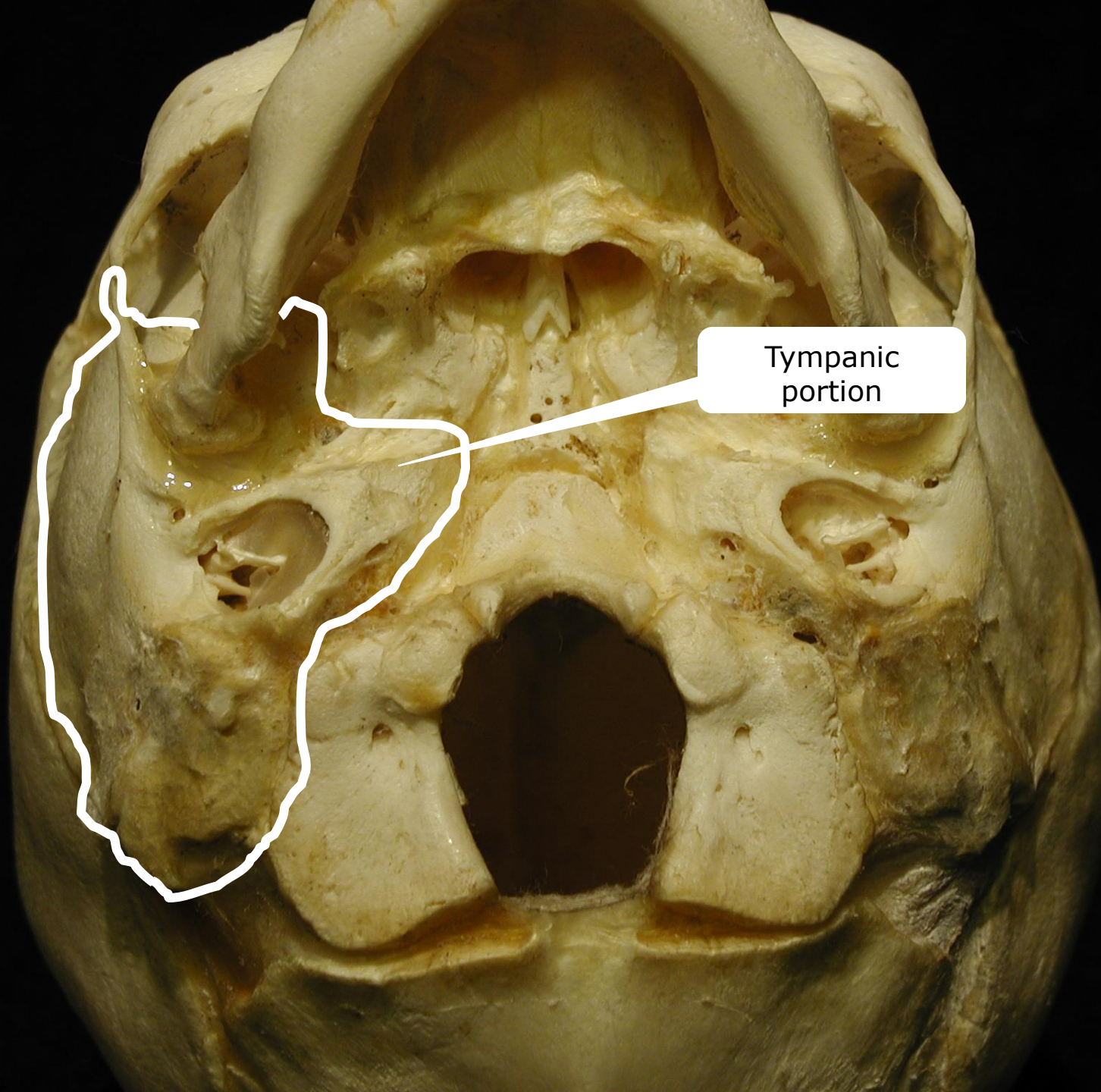
Squamous
portion

Zygomatic
portion

Mastoid
portion

Tympanic
portion

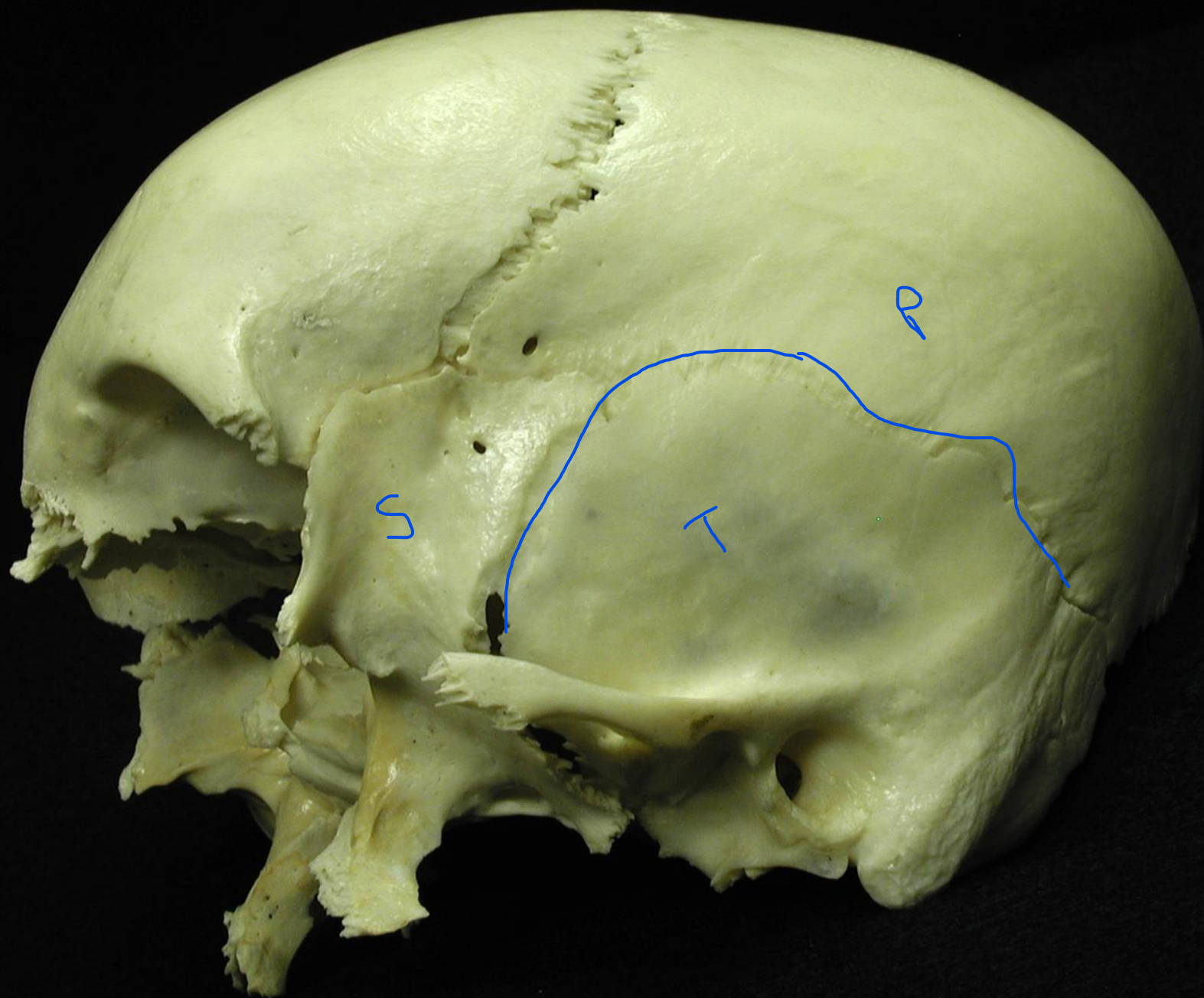
mastoid elongates with compression and tension on SCM

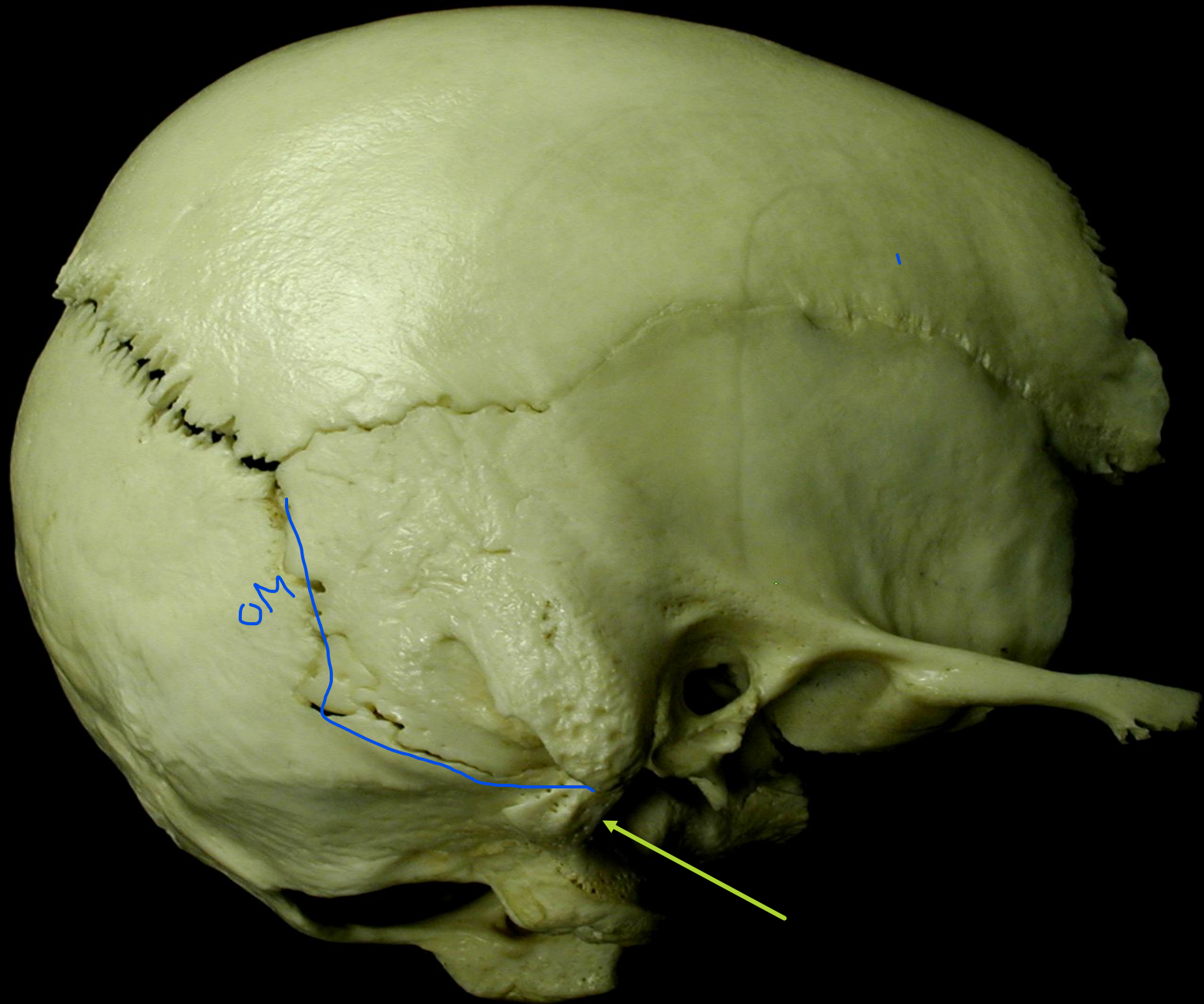


Tympanic
portion

Temporal Bone Articulations

- ▶ Occiput
- ▶ Parietal
- ▶ Sphenoid
- ▶ Zygoma
- ▶ Mandible

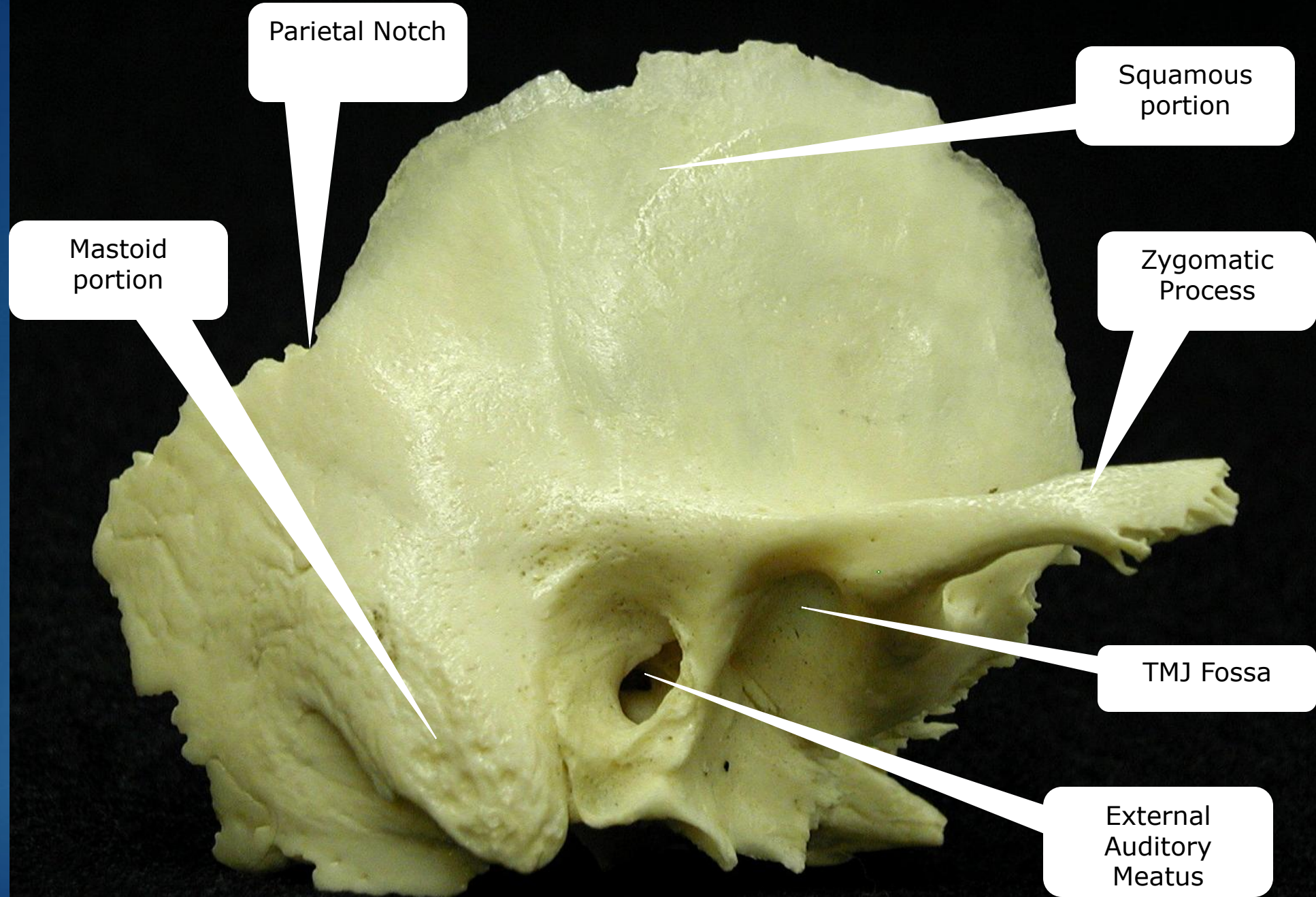






Adult Temporal Bone – Posterior, Medial & Anterior Features

- ▶ Petrous Ridge
- ▶ Parieto-Squamous & Parieto-mastoid Articulations
- ▶ Sphenosquamous Pivot (SS Pivot)
- ▶ Hiatus Fallopii (Petrosal Canal)
- ▶ Bony Eustachian Tube
- ▶ Tensor Tympani Canal
- ▶ Trigeminal Impression (Meckels Cave)
- ▶ Carotid Canal
- ▶ Sigmoid Sinus Impression
- ▶ Superior Petrosal Sinus Impression
- ▶ Internal Auditory Meatus
- ▶ Vestibular Aqueduct



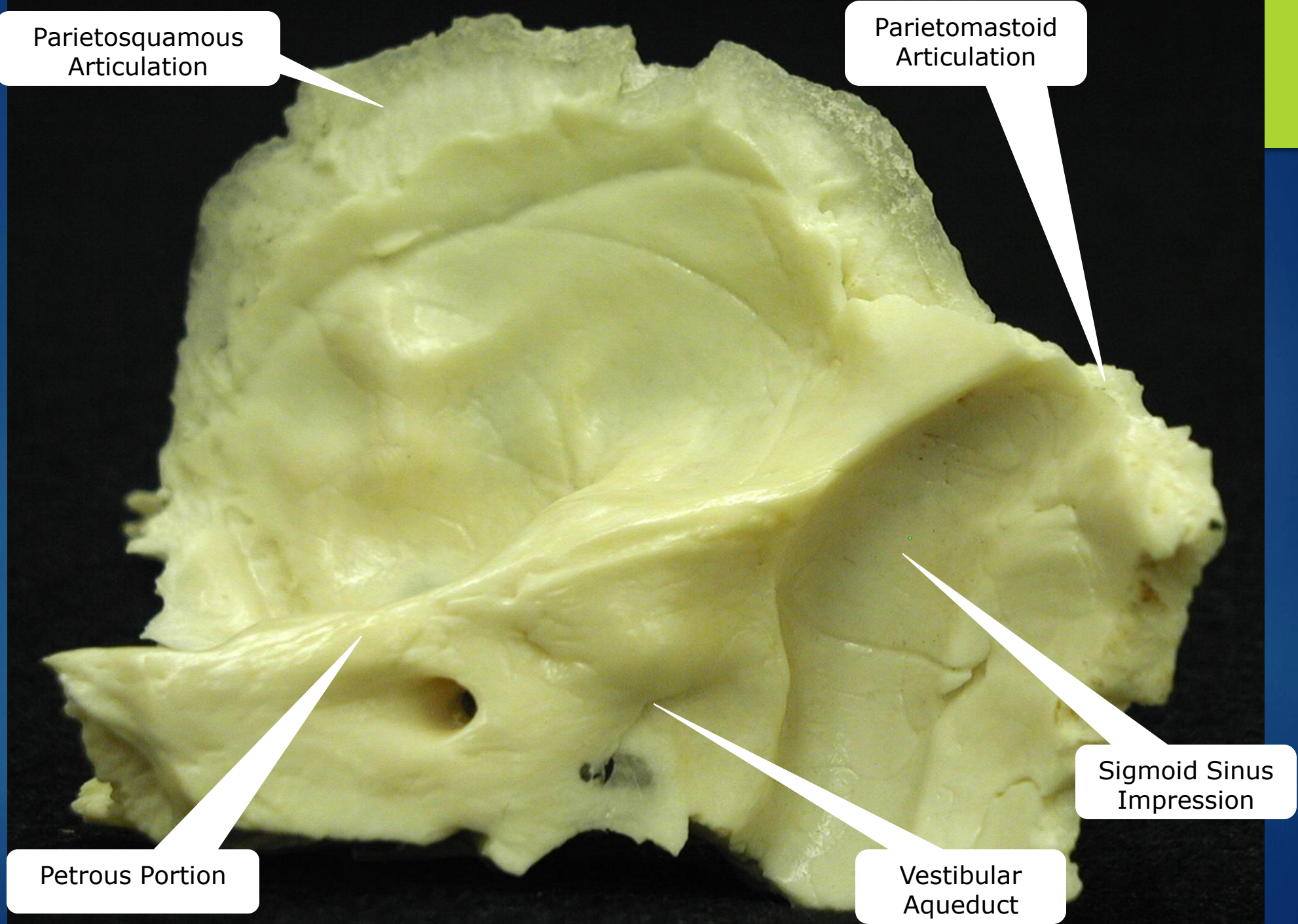
Parietosquamous
Articulation

Parietomastoid
Articulation

Petrous Portion

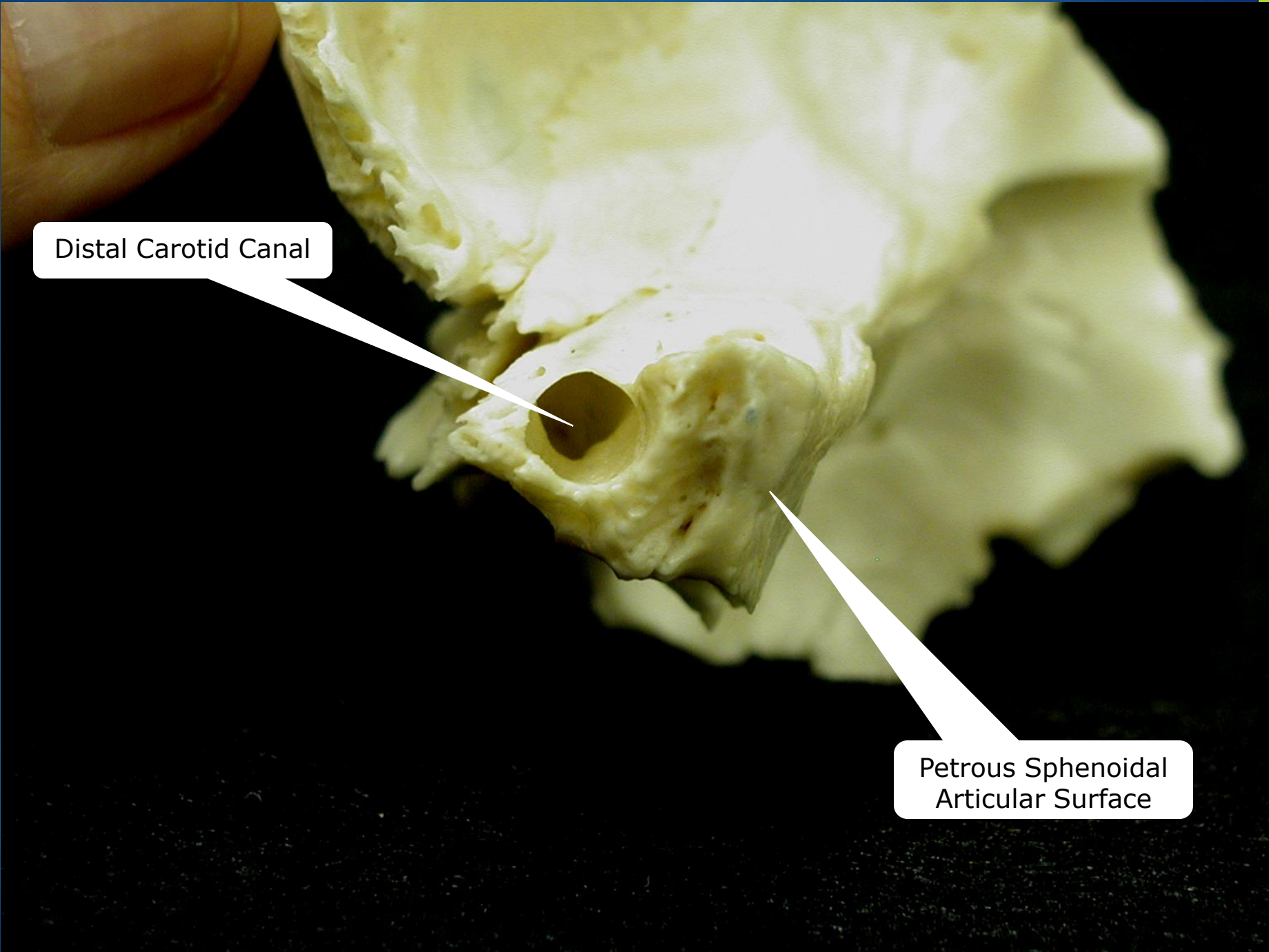
Vestibular
Aqueduct

Sigmoid Sinus
Impression



Sphenosquamous
(SS) Pivot

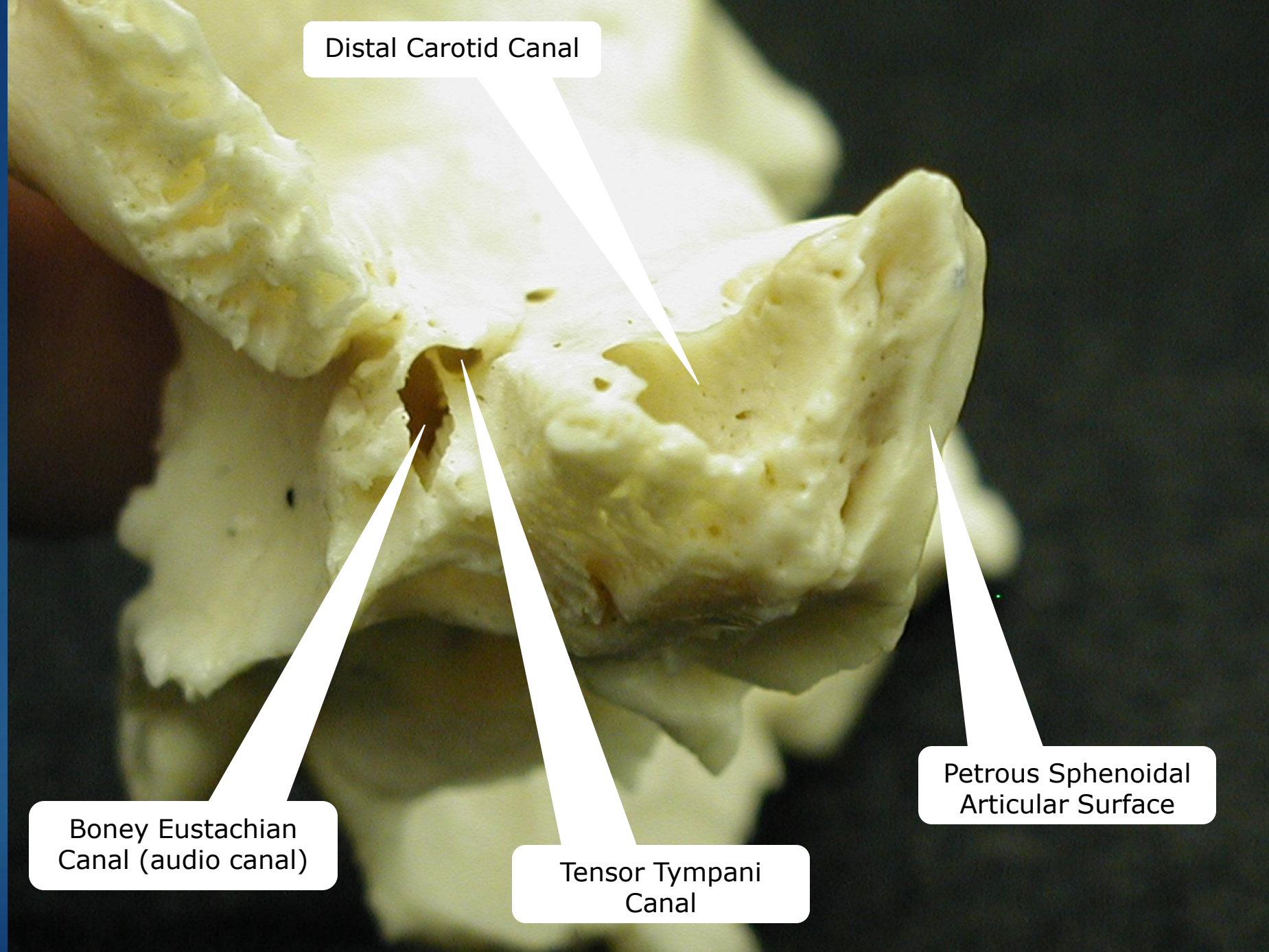




A close-up photograph of a human skull base, specifically the petrous part of the temporal bone. The image shows the distal carotid canal, a large, circular opening in the bone. A white line points from the text label to this canal. Another white line points from the text label to the petrous sphenoidal articular surface, a rough, irregular surface on the bone. The bone is light yellow and has a porous, trabecular appearance. The background is dark blue.

Distal Carotid Canal

Petrous Sphenoidal
Articular Surface

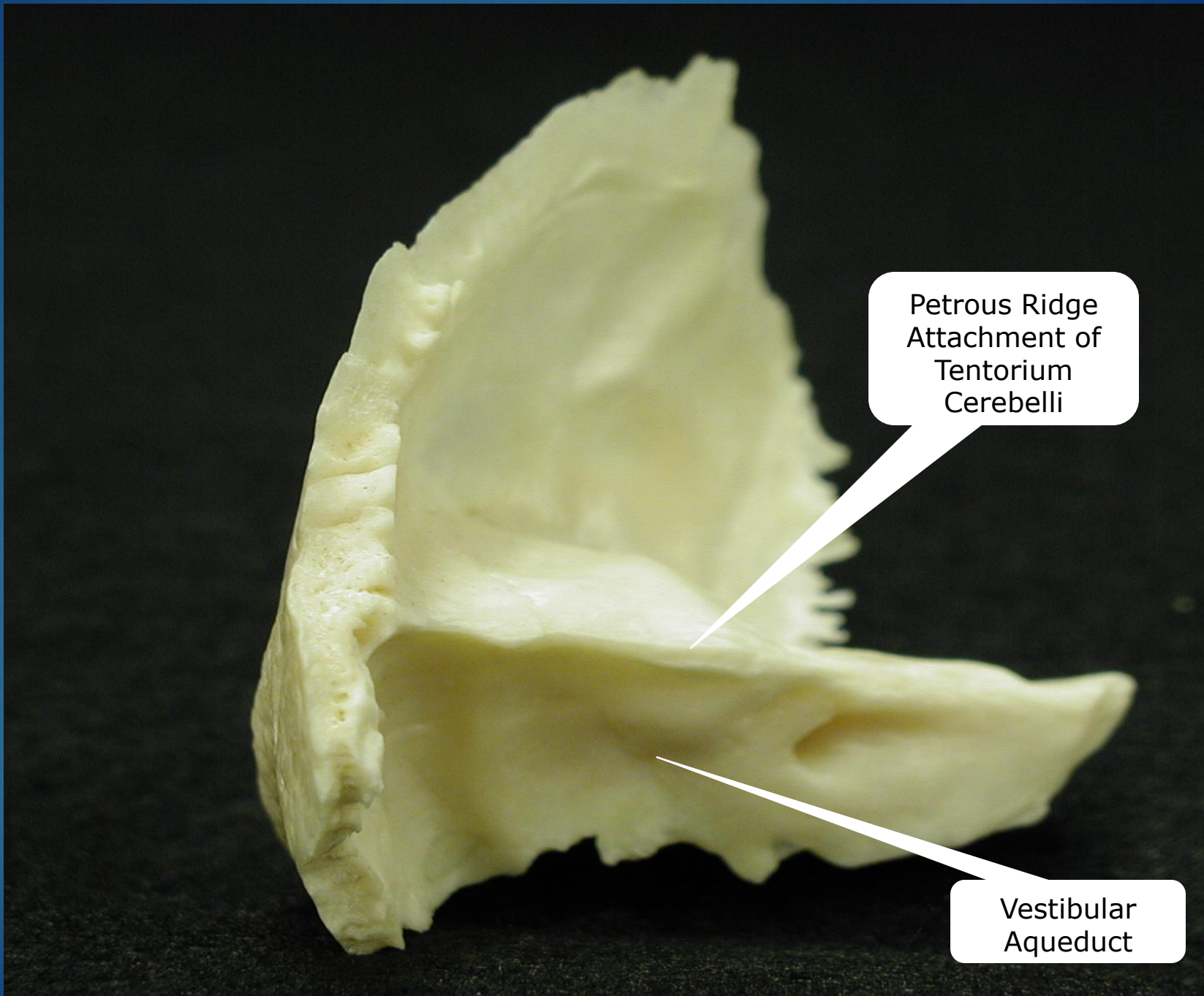


Distal Carotid Canal

Boney Eustachian
Canal (audio canal)

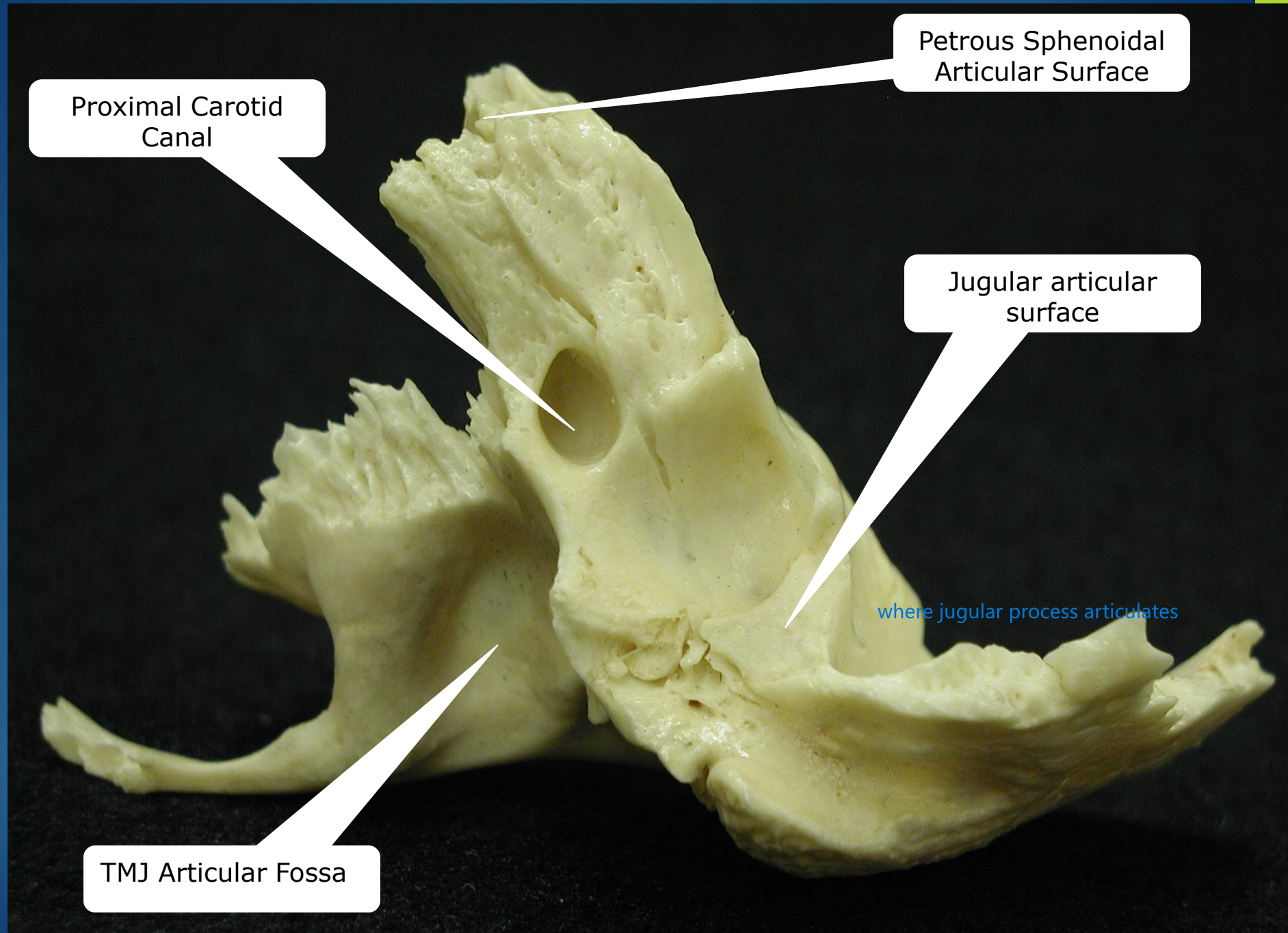
Tensor Tympani
Canal

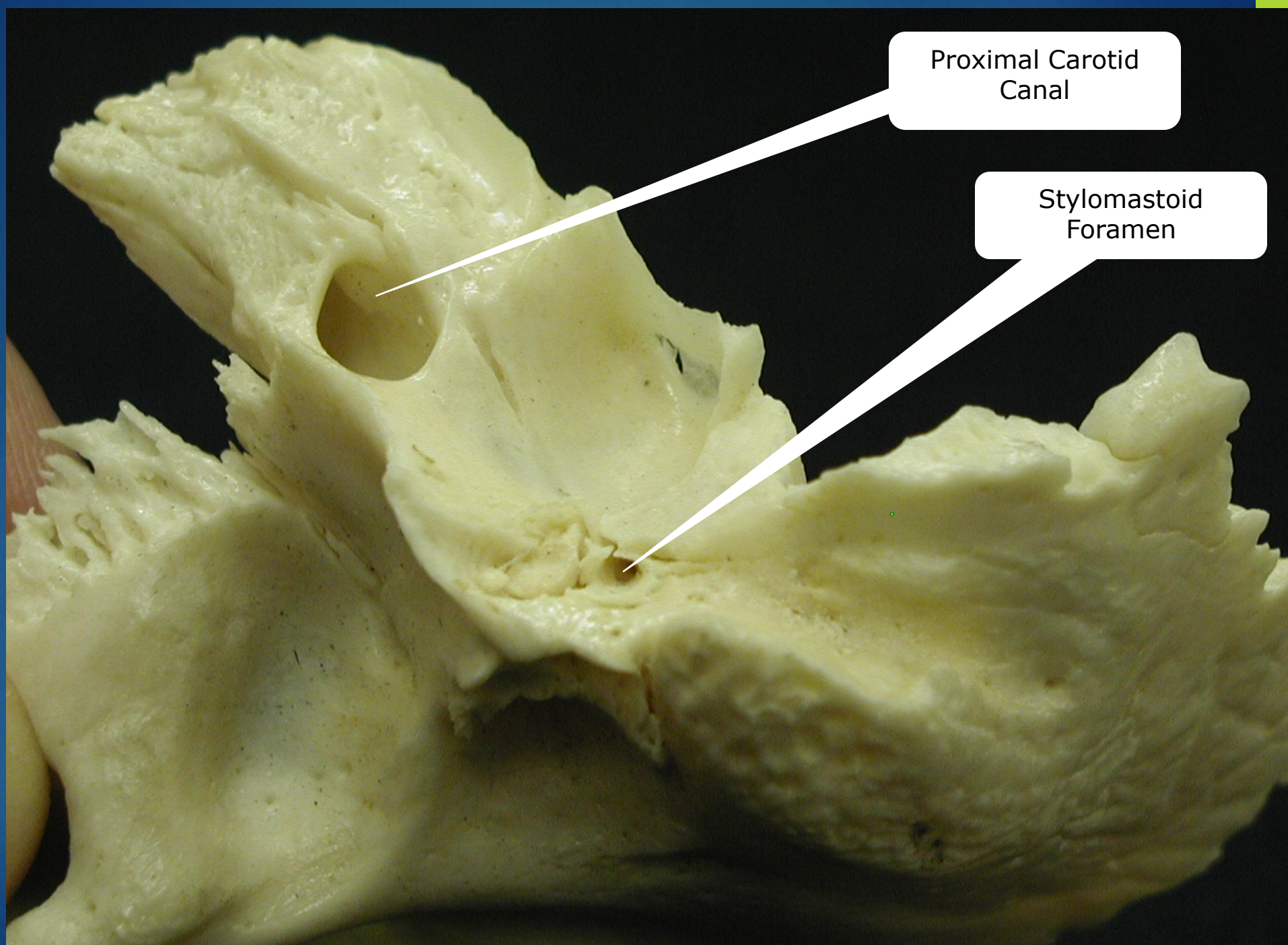
Petrous Sphenoidal
Articular Surface



Petrous Ridge
Attachment of
Tentorium
Cerebelli

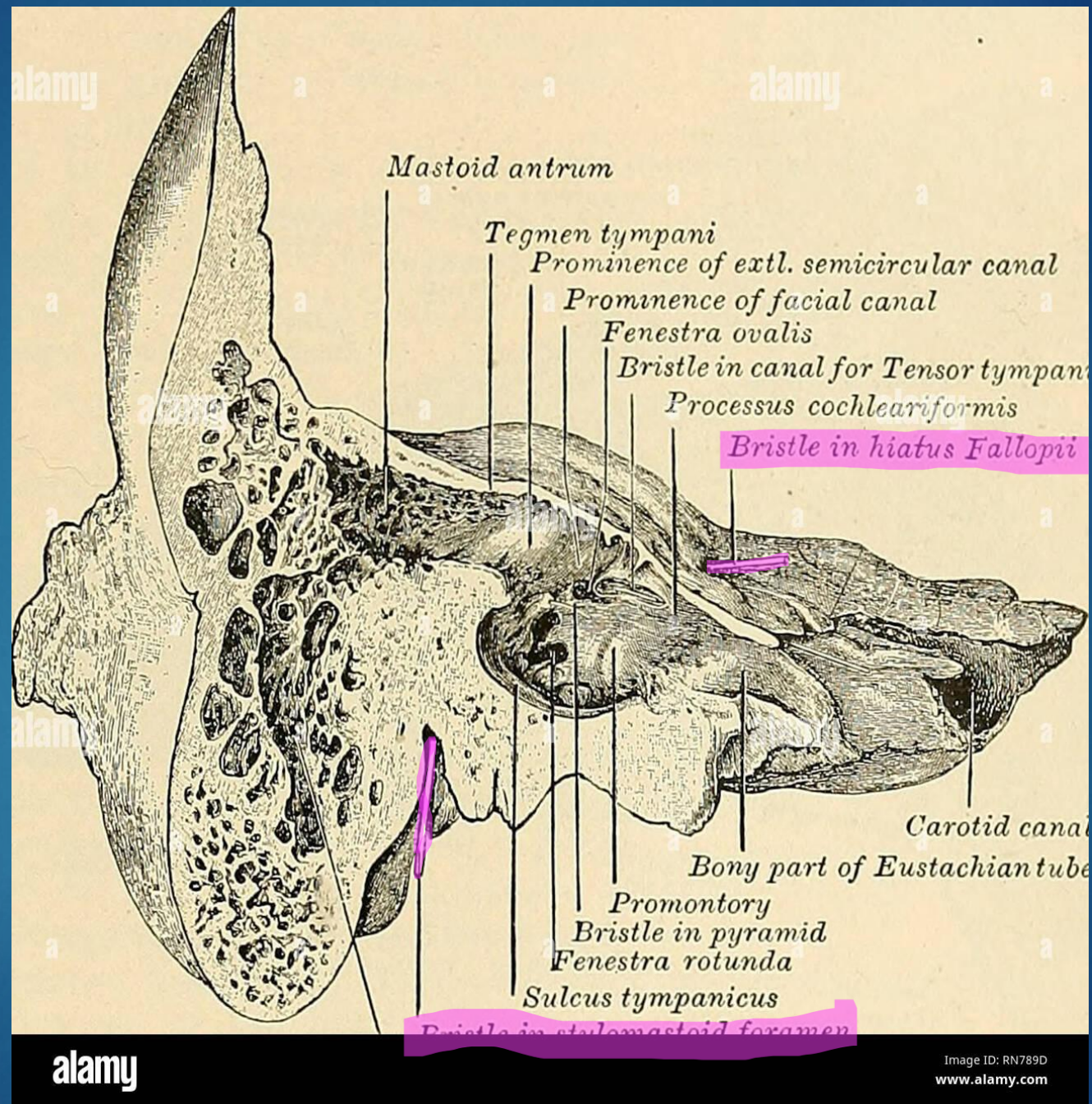
Vestibular
Aqueduct



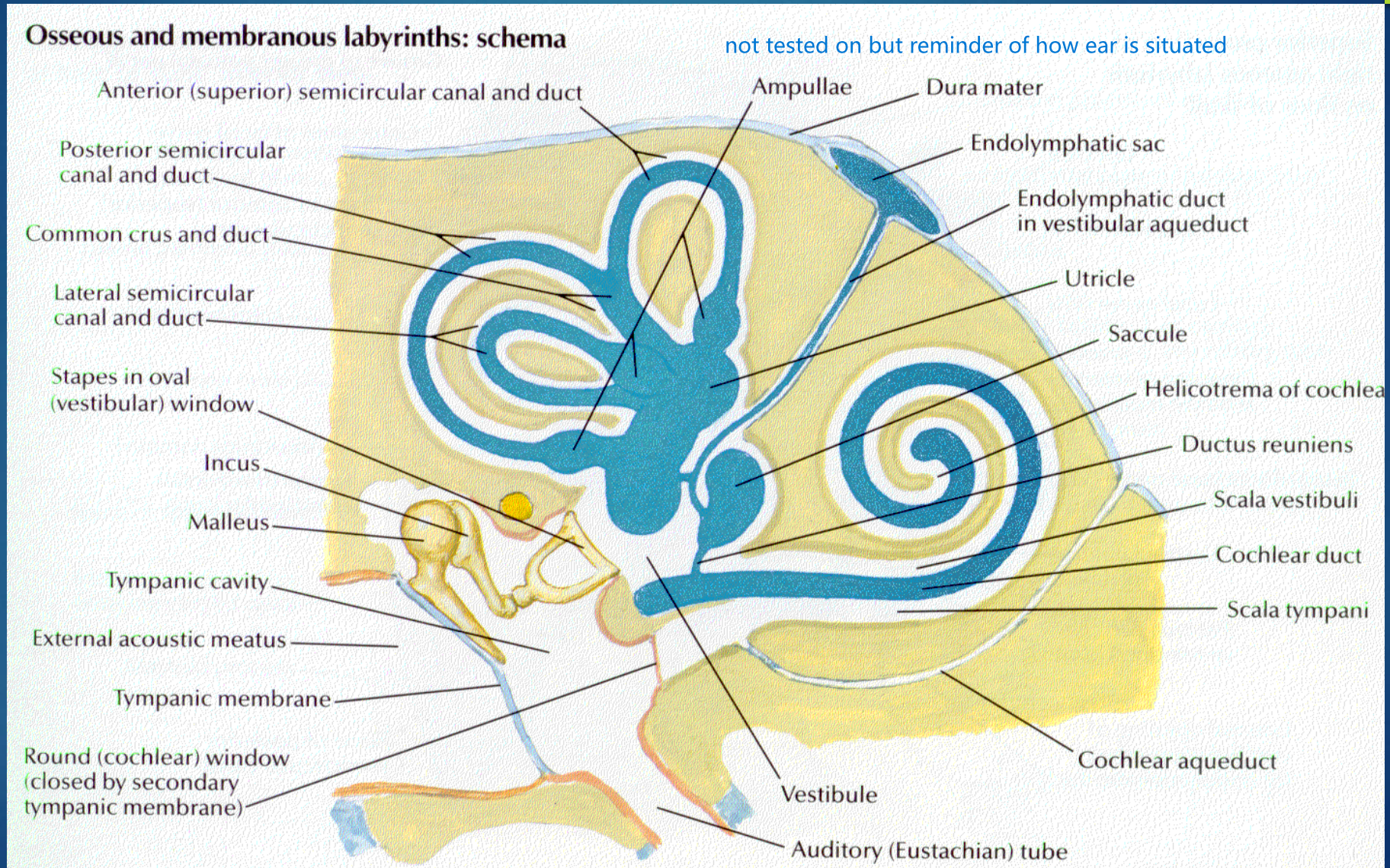


Proximal Carotid
Canal

Stylomastoid
Foramen



Cross section of Petrous Portion of the Temporal Bone



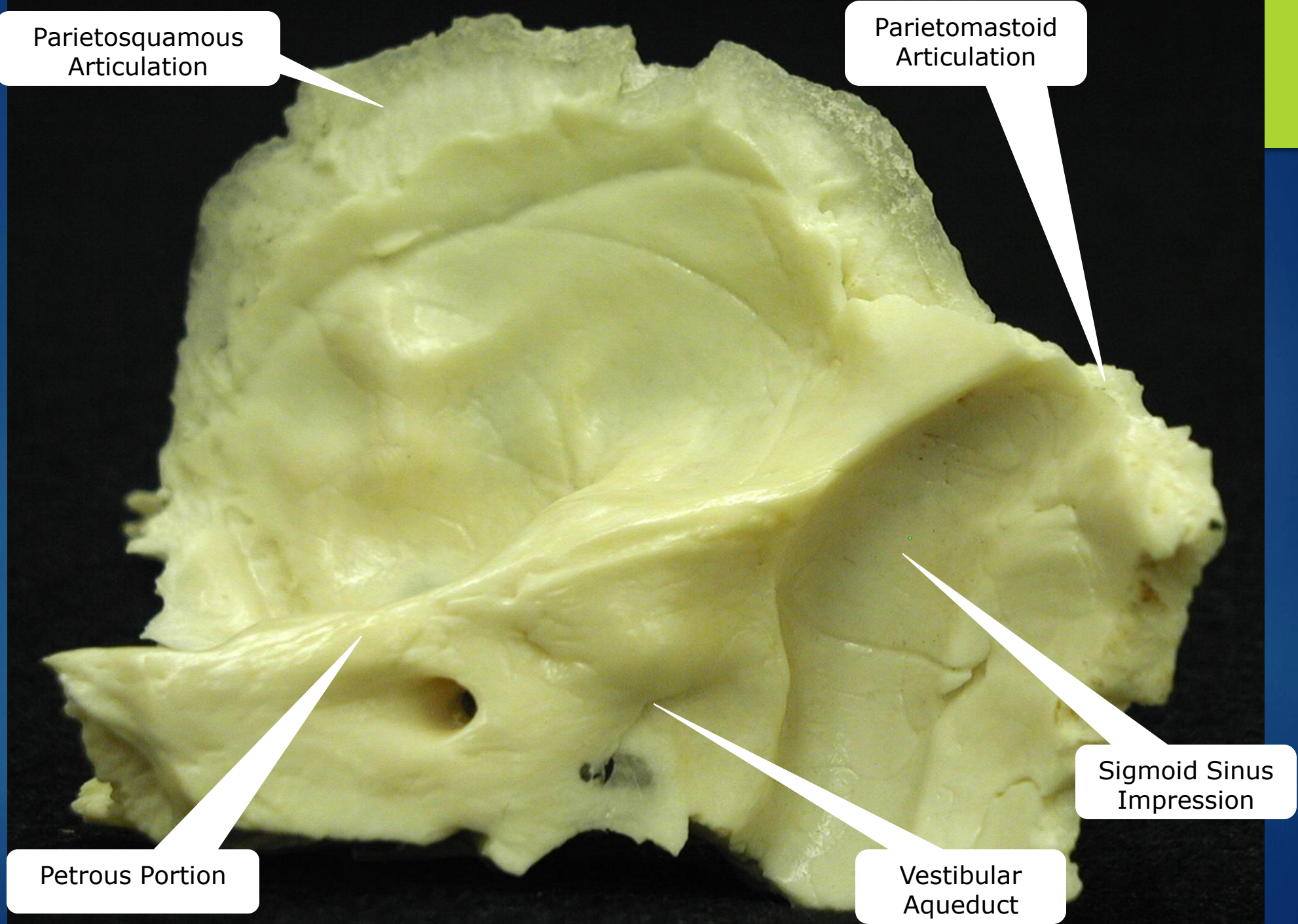
Parietosquamous
Articulation

Parietomastoid
Articulation

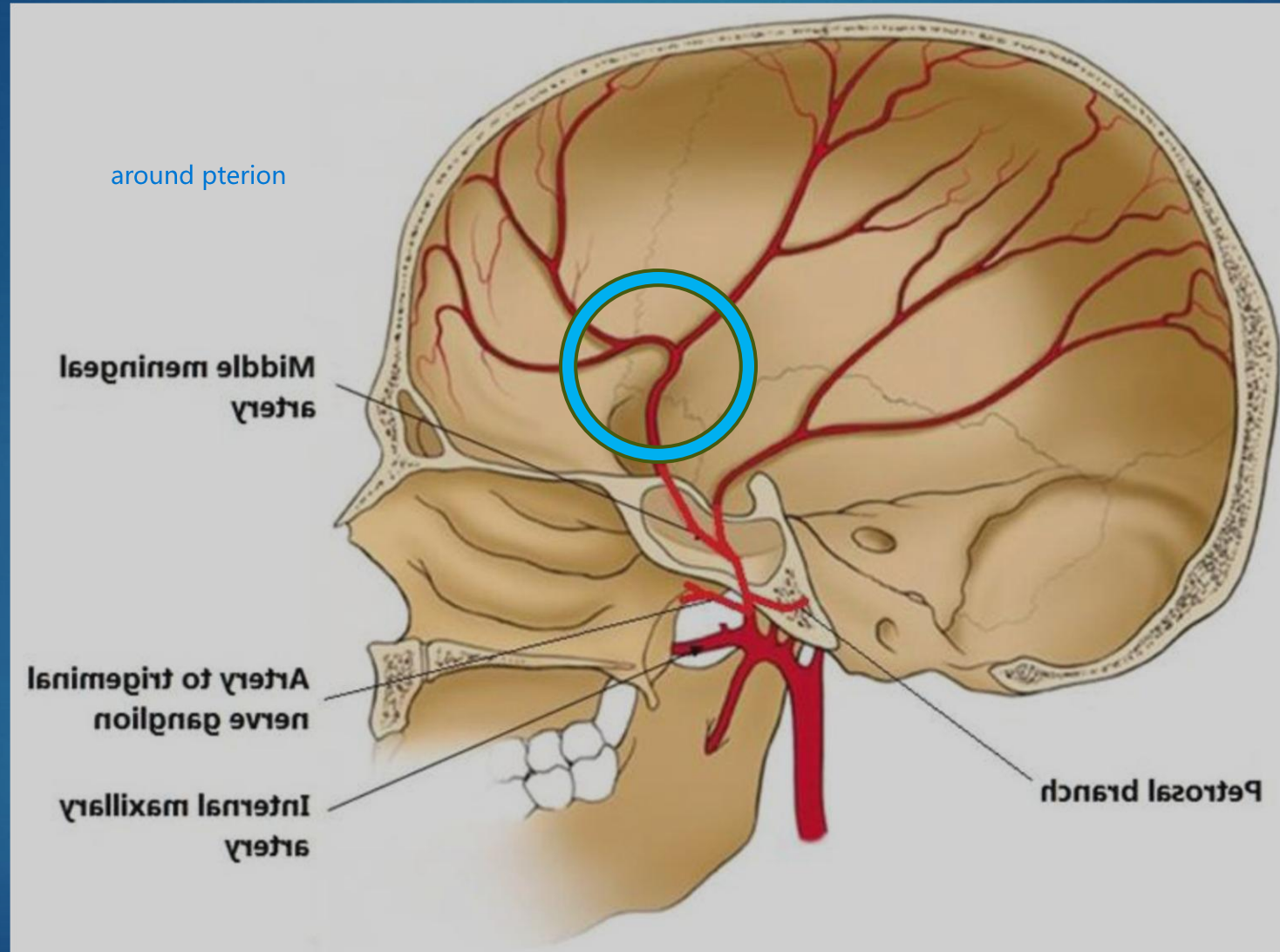
Petrous Portion

Vestibular
Aqueduct

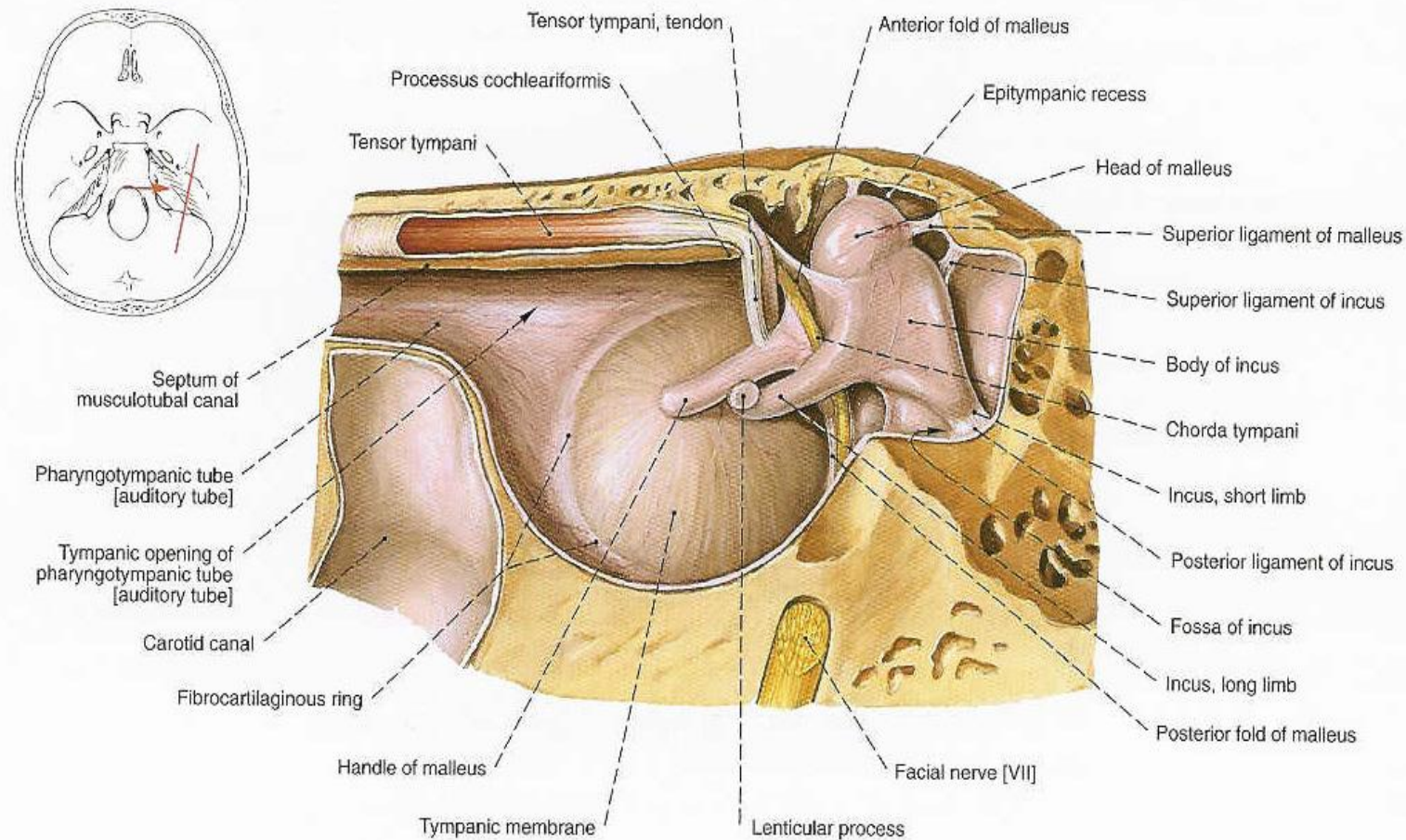
Sigmoid Sinus
Impression



Middle Meningeal Artery



Tensor Tympani Articular Relationship



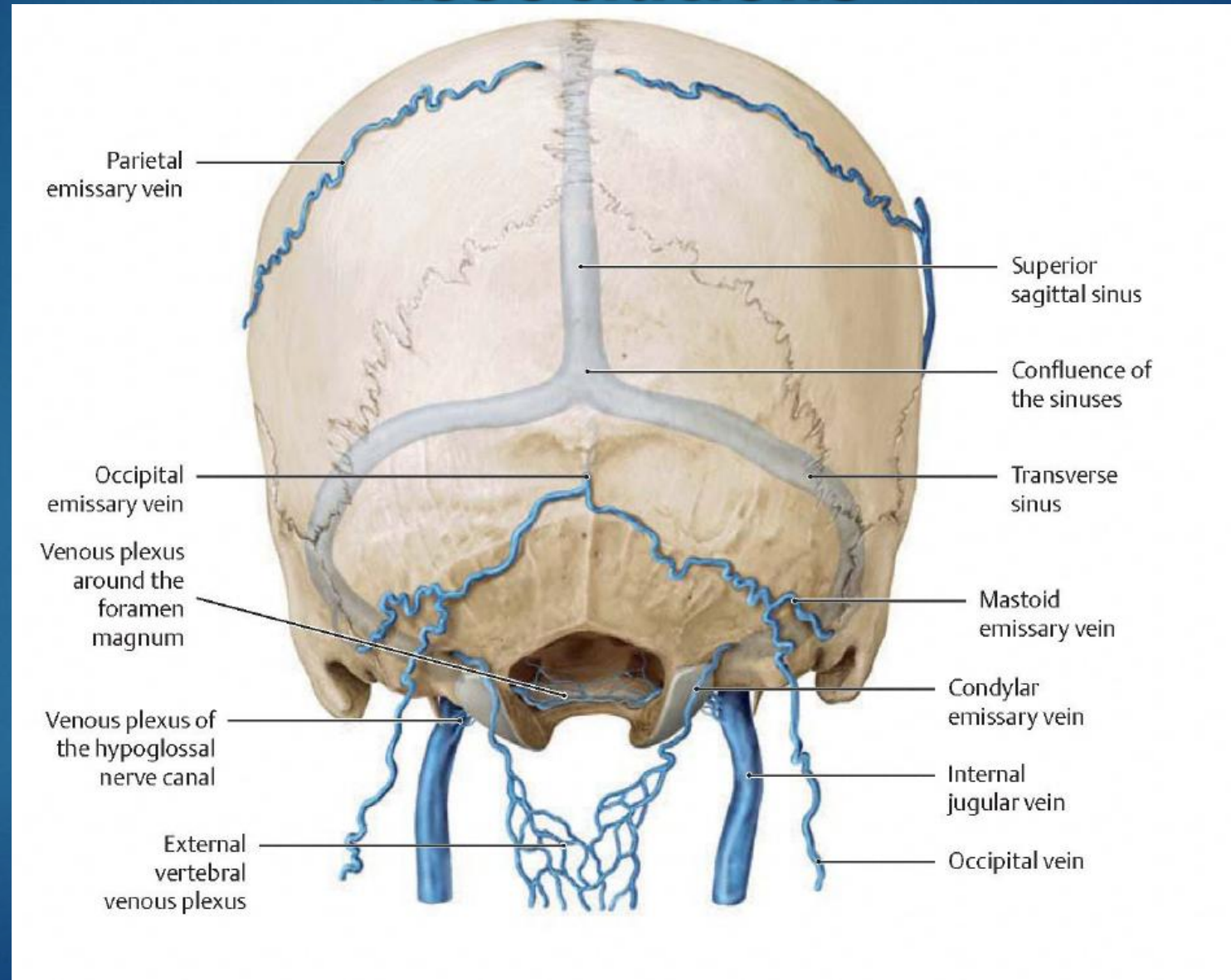
attenuates sound

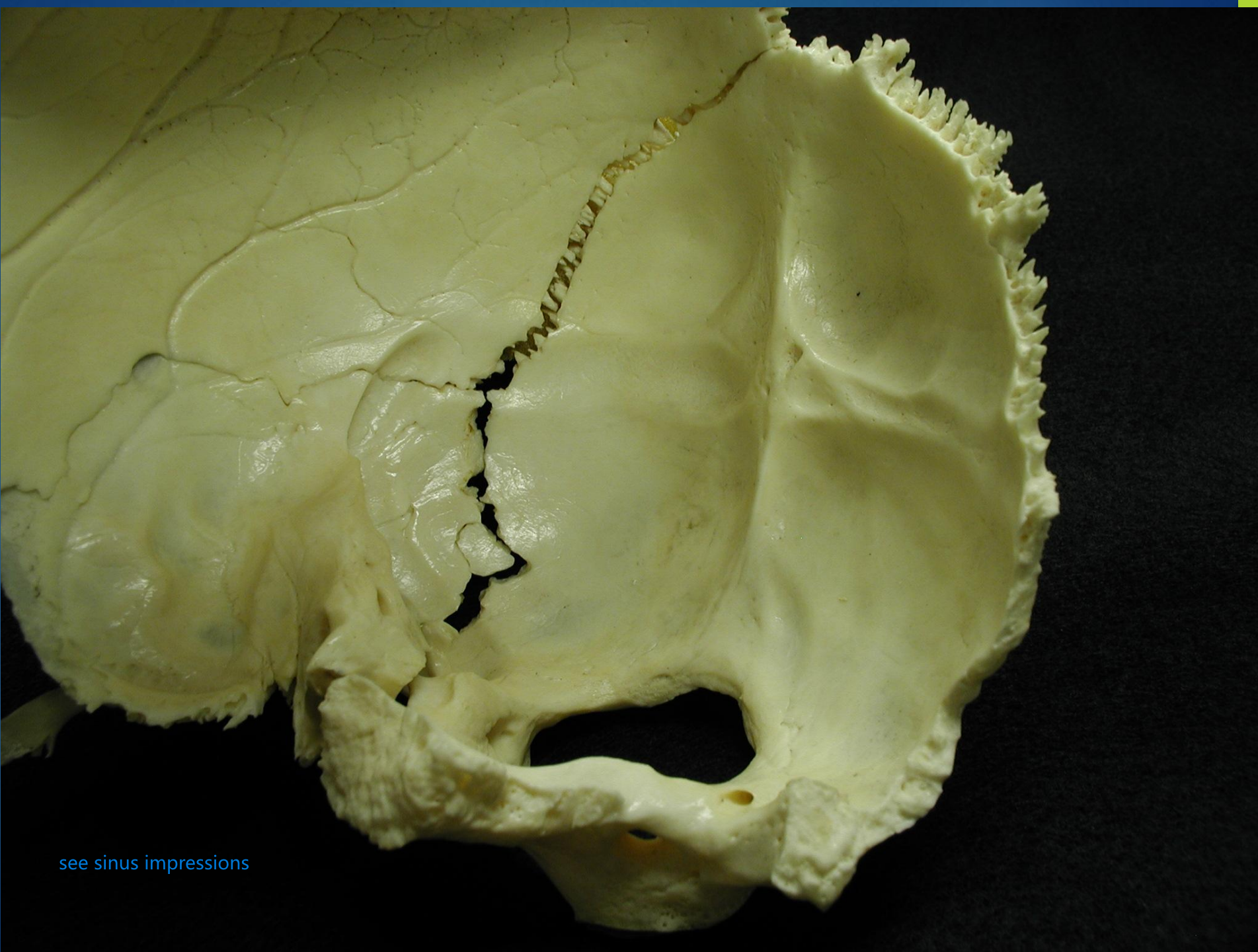
Fig. 677 Membranous wall [lateral wall] of right tympanic cavity; coronal section after removal of most of the pharyngotympanic tube [auditory tube] and removal of the fascia of tensor tympani; medial aspect (400%).

Temporal Bone Vascular Associations

- ▶ Sigmoid Sinus
- ▶ Superior Petrosal Sinus
- ▶ Inferior Petrosal Sinus
- ▶ Jugular Vein (Occipital-Mastoid Foramen)
- ▶ Internal Carotid Artery

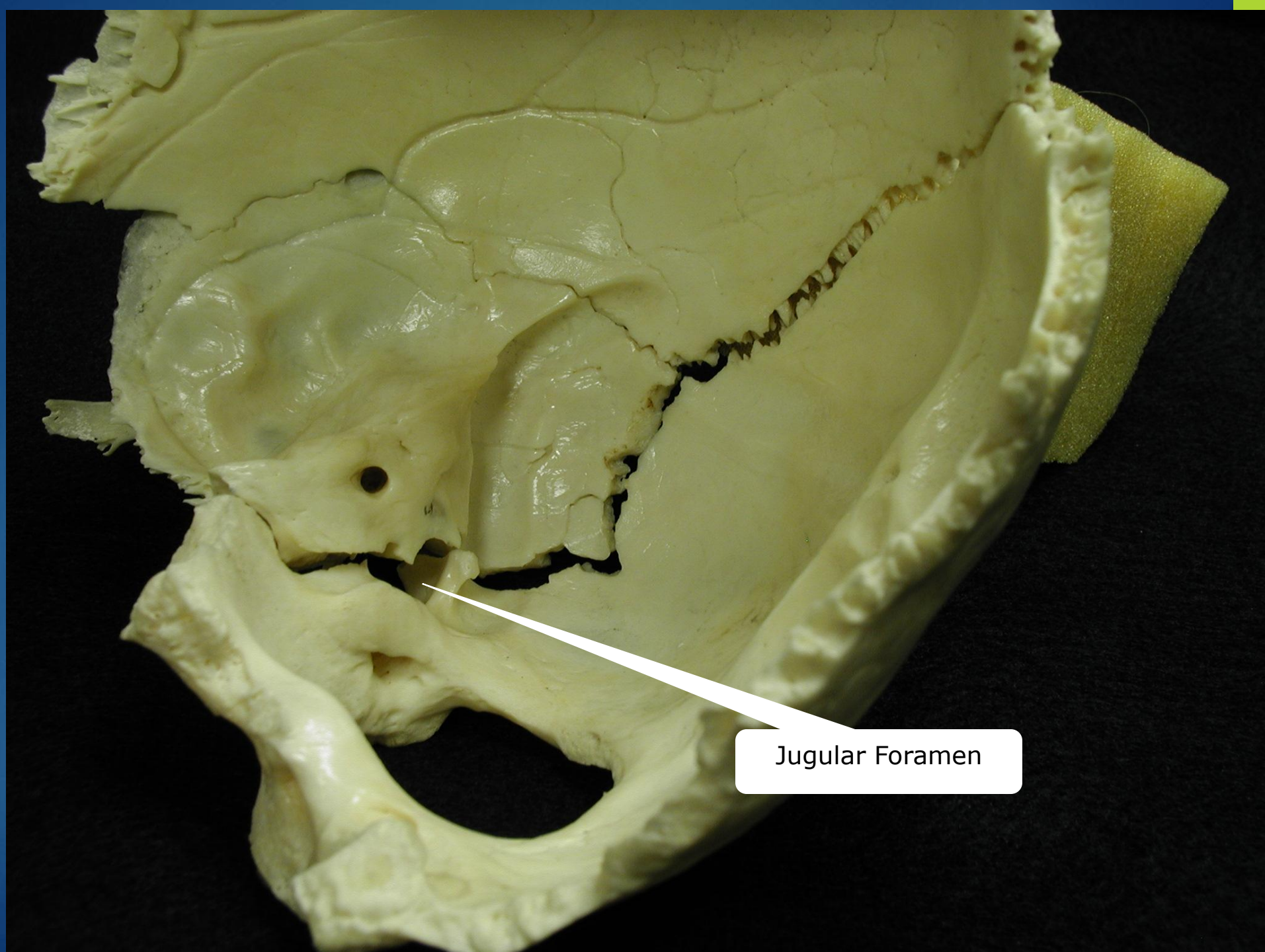
Temporal Bone Venous Drainage Associations





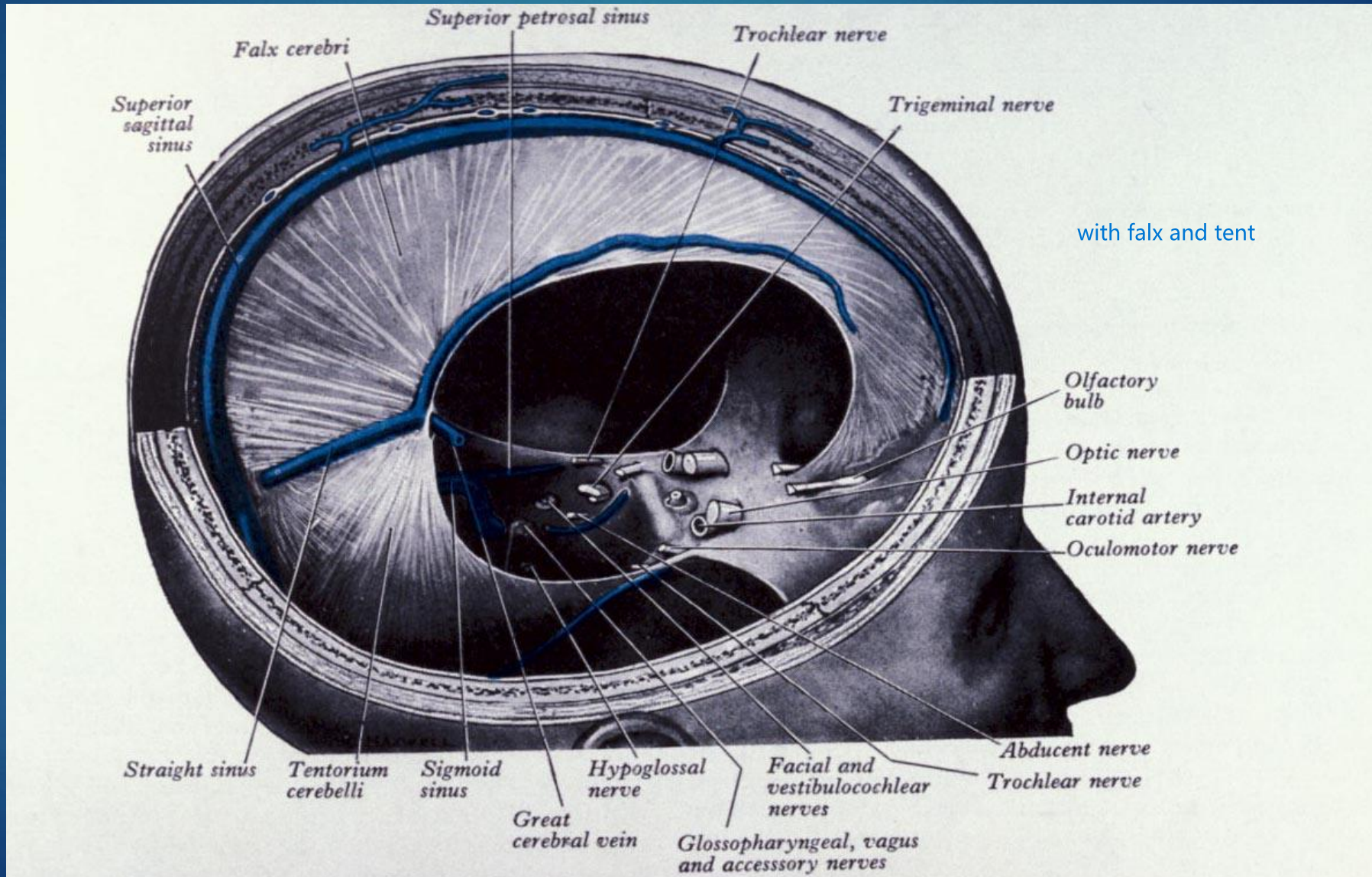
see sinus impressions

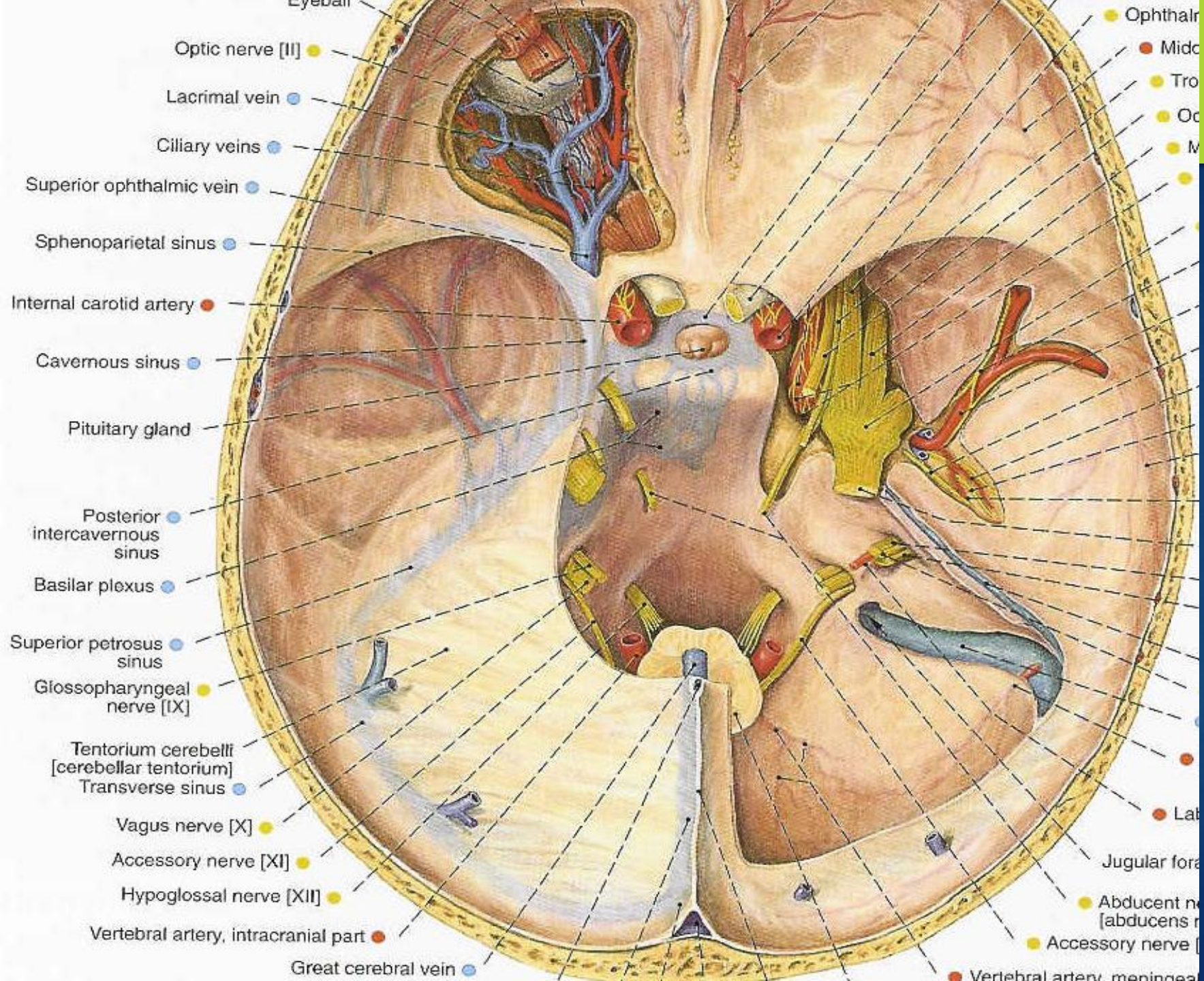


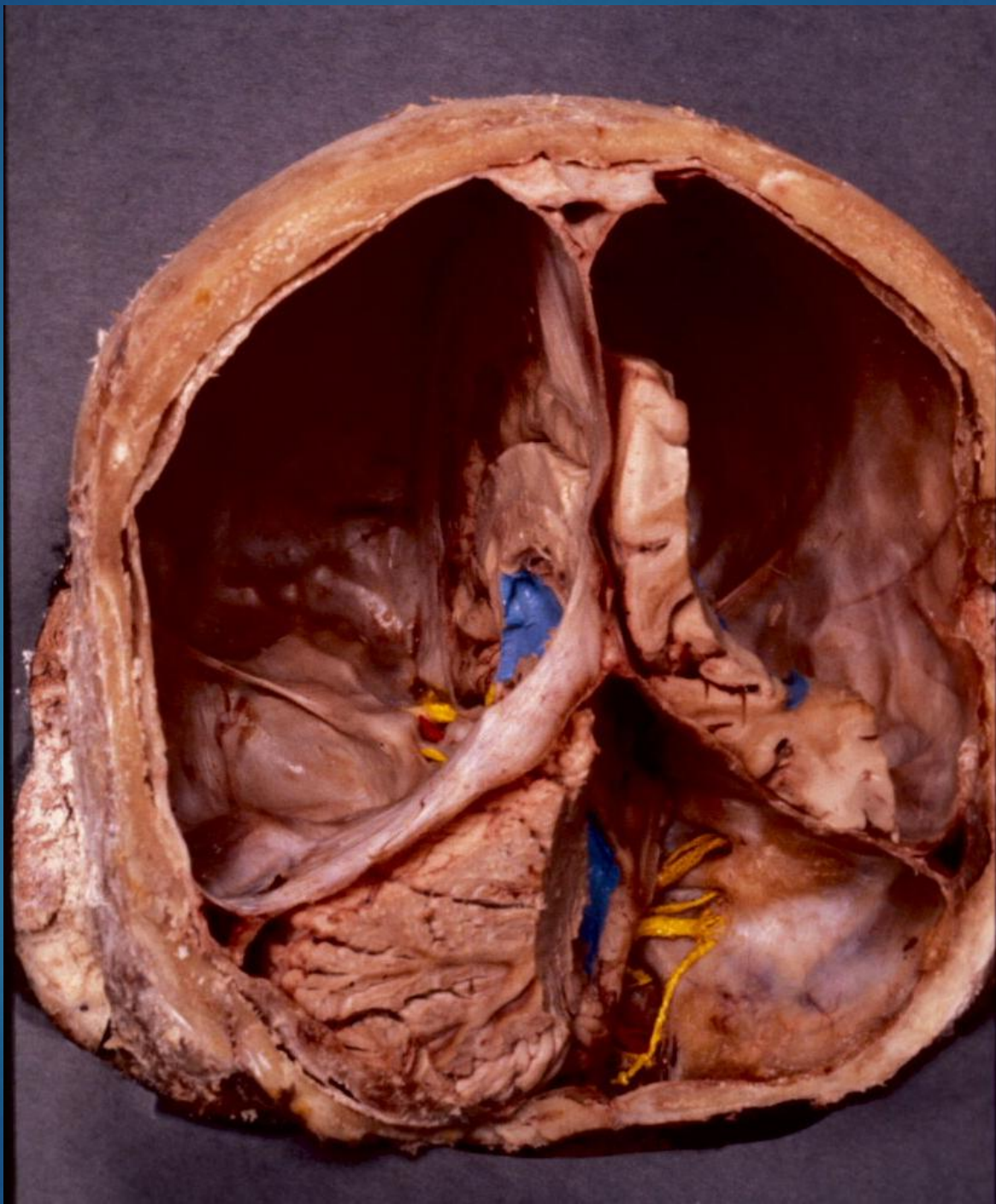


Jugular Foramen

Temporal Bone Dural Associations







Temporal Bone Neural Associations

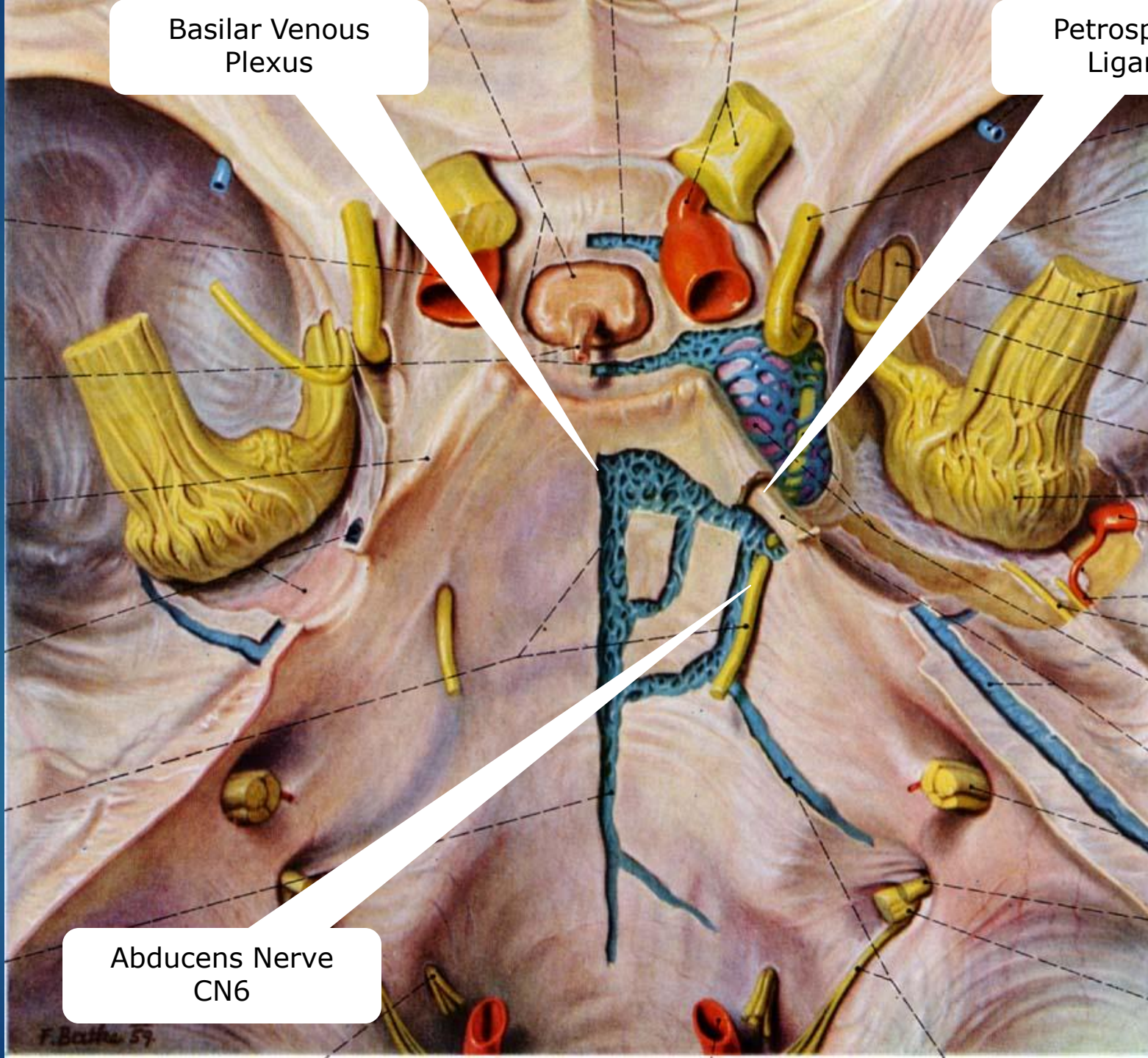
- ▶ V (Trigeminal-ganglion portion)
- ▶ VI (Abducent)
- ▶ VII (Facial)
- ▶ VIII (Vestibulo-cochlear)
- ▶ IX (Glossopharyngeal)
- ▶ X (Vagus)
- ▶ XI (Spinal Accessory)
- ▶ Greater Petrosal

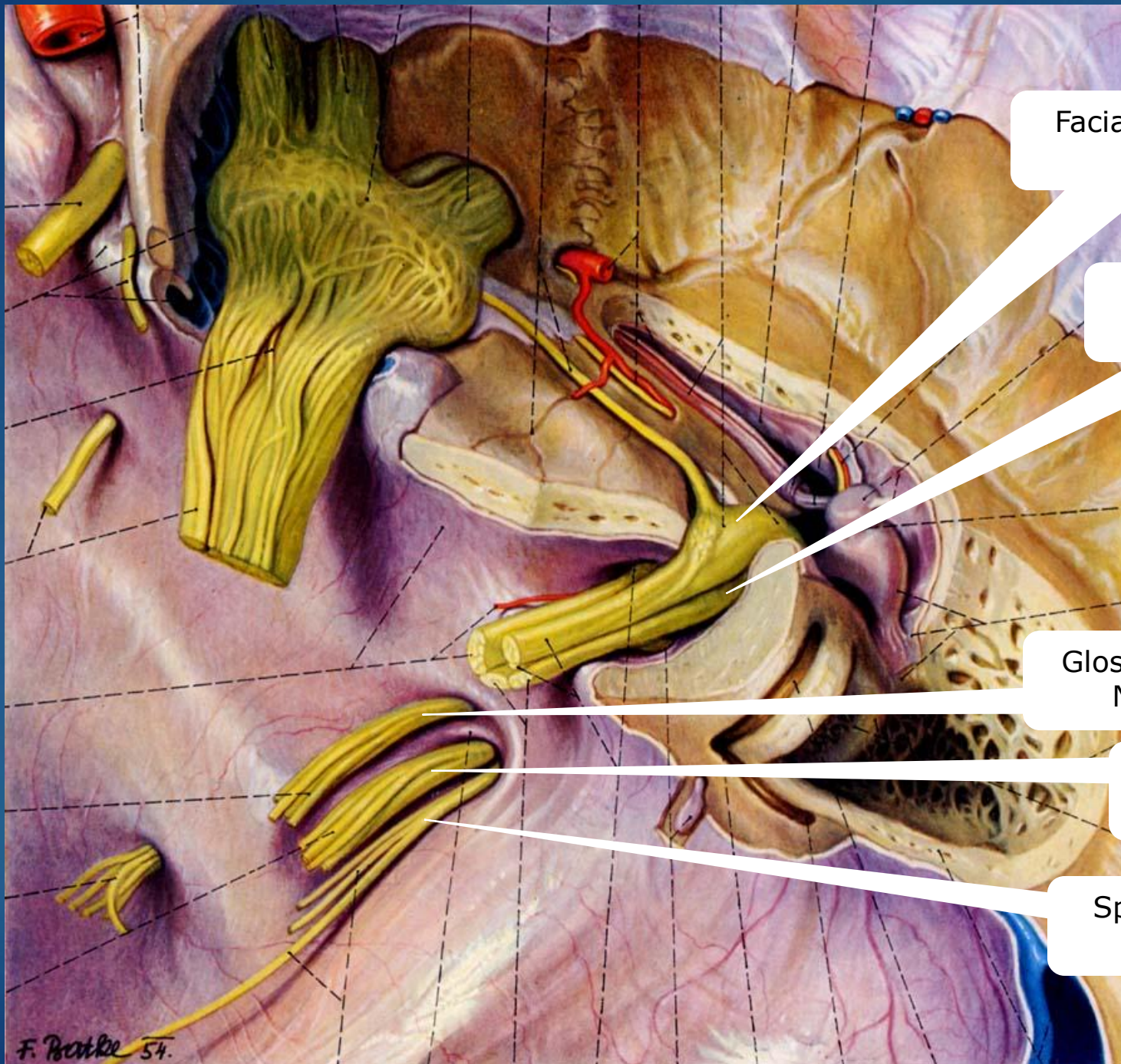
Occipital-
Mastoid
Foramen

Basilar Venous
Plexus

Petrosphenoid
Ligament

Abducens Nerve
CN6





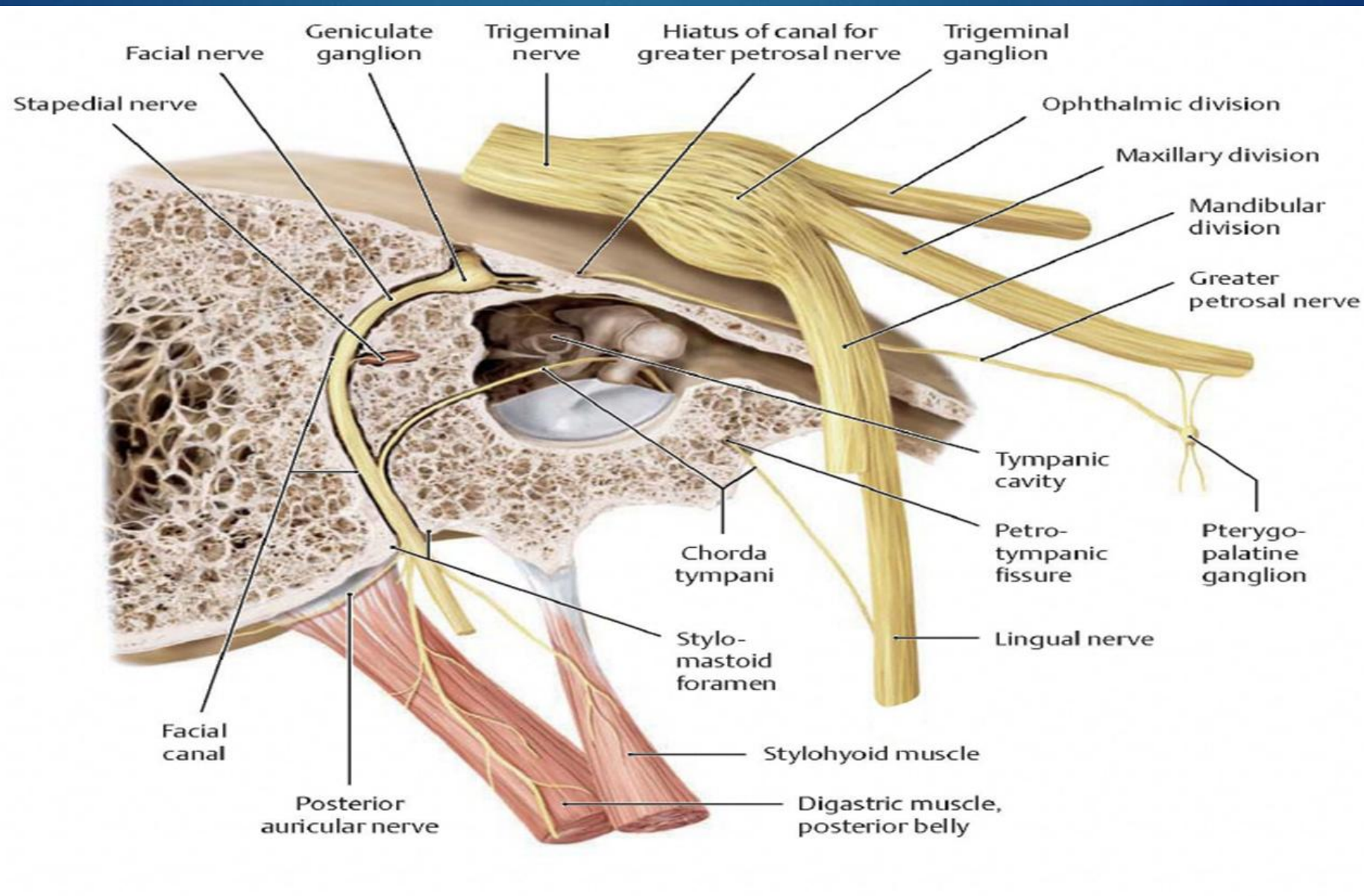
Facial Nerve CN7

Vestibulocochlear
Nerve CN8

Glossopharyngeal
Nerve CN9

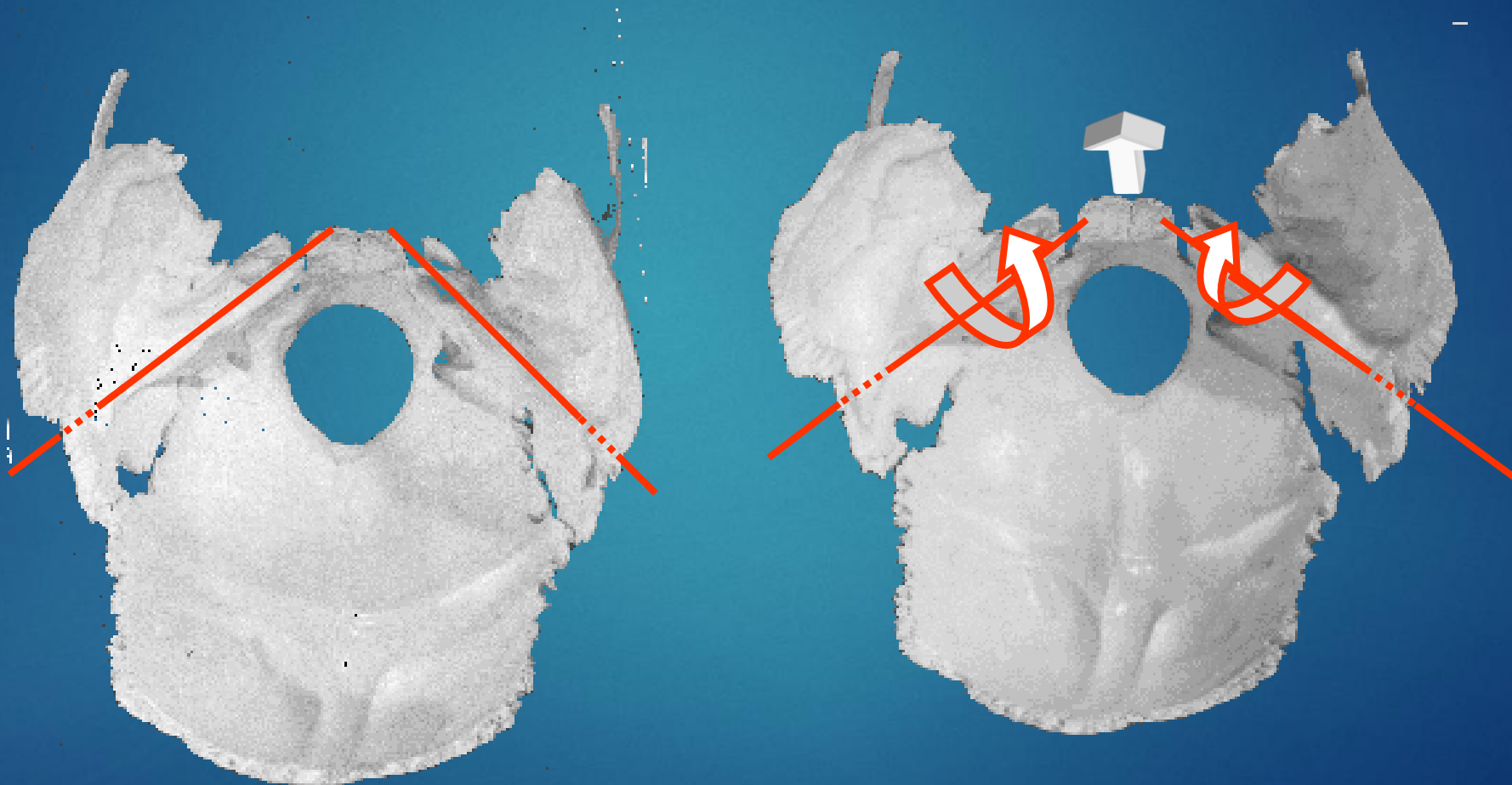
Vagus Nerve CN10

Spinal Accessory
Nerve CN11



Temporal Bone PRM Motion

Paired Bone Motion



axis around 3 digit

Clinical Indications for Temporal Bone Evaluation and/or Treatment



- ▶ Otitis (especially otitis media)
- ▶ Eustachian Tube Dysfunction
- ▶ Vertigo
- ▶ Tinnitus
- ▶ Hyper/Hypoacusis
- ▶ GI Autonomic Dysfunction
- ▶ CN7 palsy (Bell's Palsy)
- ▶ Migraine cephalgia
- ▶ Extraocular muscle dysfunction
- ▶ Secondary Respiratory Compromise
- ▶ TMJ Dysfunction

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